



MCFD RESIDENTIAL CARE POLICY MANUAL

Standards for Staffed Children's Residential Services

In Accordance with the Child, Family and Community Service Act (CFCSA)

Community Care and Assisted Living Act (CCALA) & Residential Care Regulation (RCR)

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Applies To	All staff, volunteers, contractors, and students in Evergreen Community Care residential programs; all children and youth placed by MCFD



TABLE OF CONTENTS

1. Legislative and Regulatory Framework
2. Rights of Children and Youth in Care
- STANDARD A — PURPOSE AND FUNCTION**
3. Standard A — Purpose and Function of the Program
- STANDARD B — RIGHTS OF CHILDREN AND YOUTH**
4. Standard B — Rights, Complaint Resolution, and Privacy
- STANDARD C — PERSONAL SAFETY**
5. Standard C — Personal Safety of Children and Youth
- STANDARD D — QUALITY OF SERVICE EXPERIENCES**
6. Standard D — Quality of Service Experiences (D.1–D.8)
- STANDARD E — CHILD AND YOUTH SERVICE PLANNING**
7. Standard E — Child and Youth Service Planning (E.1–E.7)
- STANDARD F — SHELTER AND EQUIPMENT**
8. Standard F — Shelter and Equipment (F.1–F.4)
- STANDARD G — HUMAN RESOURCES**
9. Standard G — Human Resources (G.1–G.4)
- STANDARD H — ORGANIZATION AND MANAGEMENT**
10. Standard H — Organization and Management (H.1–H.2)
- SUPPLEMENTARY POLICIES**
11. Indigenous Reconciliation Framework and Cultural Safety
12. Gender-Affirming Care Policy
13. Missing Child / Youth Protocol
14. Policy Review and Approval
15. Signatures and Approval

1. LEGISLATIVE AND REGULATORY FRAMEWORK

1.1 Primary Legislation

All Evergreen Community Care residential services are delivered within the authority and requirements of the following legislative framework:

Legislation / Regulation	Key Application to Residential Care
Child, Family and Community Service Act (CFCSA, 1996)	Governs all child welfare services in BC; establishes Guiding Principles, Service Delivery Principles, Best Interests of the Child, and Rights of Children in Care (s.70); mandatory duty to report (s.14)
Community Care and Assisted Living Act (CCALA)	Governs licensing of community care facilities in BC; requires a valid license to operate
Residential Care Regulation (RCR, last amended April 1, 2026)	Sets detailed physical, operational, and staffing standards for licensed residential care facilities
Child in Care Regulation	Establishes specific rights and standards for children in government care
Declaration on the Rights of Indigenous Peoples Act (DRIPA)	Implements UNDRIP in BC; governs Indigenous child and family services; guides Indigenous placement decisions and Cultural Planning
BC Human Rights Code	Prohibits discrimination; establishes rights related to gender identity, sexual orientation, disability, and all protected grounds for children, youth, and staff
Freedom of Information and Protection of Privacy Act (FOIPPA)	Governs collection, use, and disclosure of personal information in individual service records
Criminal Records Review Act	Governs criminal record check requirements for all staff and volunteers with access to children
Workers Compensation Act / OHS Regulation	Governs workplace health and safety for all staff
Motor Vehicle Act	Governs vehicle safety requirements when transporting children and youth

1.2 CFCSA Guiding Principles

The following Guiding Principles from section 2 of the CFCSA form the basis for all practice and decision-making at Evergreen Community Care:

- The safety and well-being of children are the paramount considerations
- Children are entitled to be protected from abuse, neglect, and harm or threat of harm
- A family is the preferred environment for the care and upbringing of children
- Indigenous families and Indigenous communities share responsibility for the upbringing and well-being of Indigenous children
- If, with available support services, a family can provide a safe and nurturing environment, support services should be provided
- The child's views should be taken into account when decisions relating to a child are made
- Kinship ties and a child's attachment to the extended family should be preserved if possible
- The cultural identity of Indigenous children should be preserved



- Indigenous children are entitled to learn about and practise their Indigenous traditions, customs, and languages, and to belong to their Indigenous communities
- Decisions relating to children should be made and implemented in a timely manner

1.3 CFCSA Service Delivery Principles

- Families and children should be informed of services available to them and encouraged to participate in decisions that affect them
- Indigenous people should be involved in the planning and delivery of services to Indigenous families and their children
- Services should be planned and provided in ways sensitive to the needs and cultural, racial, and religious heritage of those receiving services
- The impact of residential schools on Indigenous children, families, and communities should be considered in the planning and delivery of services
- Services should be integrated, wherever possible, with services provided by other government ministries, community agencies, and CLBC
- The community should be involved, wherever possible, in the planning and delivery of services

1.4 Best Interests of the Child

All actions and decisions at Evergreen Community Care must take into consideration the best interests of each child, considering all relevant factors including:

- The child's safety
- The child's physical and emotional needs and level of development
- The importance of continuity in the child's care
- The quality of the relationship the child has with a parent or other person and the effect of maintaining that relationship
- The child's cultural, racial, linguistic, and religious heritage
- The child's views
- The effect on the child if there is delay in making a decision

For Indigenous children, additional factors must be considered:

- The importance of the child being able to learn about and practise their Indigenous traditions, customs, and languages
- The importance of the child belonging to their Indigenous community

2. RIGHTS OF CHILDREN AND YOUTH IN CARE

2.1 Legal Foundation — Section 70 CFCSA

The rights of children and youth in care are enshrined in section 70 of the Child, Family and Community Service Act as legal requirements that are enforceable. Every staff member of Evergreen Community Care must affirm and respect these rights. It is the responsibility of every caregiver to ensure services are consistent with the rights.

2.2 Rights of Every Child and Youth in Care

Under section 70 of the CFCSA, every child and youth in care has the right to:

- Be fed, clothed, and nurtured according to community standards and to be given the same quality of care as other children in the placement, including the caregiver's own children
- Be informed about their plans of care
- Be consulted and to express their views, according to their abilities, about significant decisions affecting them
- Reasonable privacy and possession of their personal belongings
- Be free from corporal punishment
- Be informed of the standard of behaviour expected by their caregivers and of the consequences of not meeting expectations
- Receive medical and dental care when required
- Participate in social and recreational activities if available and appropriate and according to their abilities and interests
- Receive the religious instruction and to participate in the religious activities of their choice
- Receive guidance and encouragement to maintain their cultural heritage
- Be provided with an interpreter if language or disability is a barrier to consulting with them on decisions affecting their custody or care
- Privacy during discussions with members of their families, subject to any court order
- Privacy during discussions with a lawyer, the Representative for Children and Youth, the Ombudsperson, a Member of the Legislative Assembly, or a Member of Parliament
- Be informed about and to be assisted in contacting the Representative for Children and Youth (RCY) or the Ombudsperson
- Be informed of their rights under the CFCSA and FOIPPA and the procedures for enforcing those rights

2.3 Additional Rights for Indigenous Children and Youth

- Receive guidance, encouragement, and support to learn about and practise their Indigenous traditions, customs, and languages
- Belong to their Indigenous communities

2.4 Staff Obligations Regarding Rights

- All staff are trained on children's and youth's rights as part of orientation, within 30 days of hire



- Rights information is provided to each child and youth upon placement, in a format understandable to them — including alternative formats, visual supports, and other languages as needed
- Staff can describe how information about rights, including how to contact the RCY, are applied in the program
- Children and youth have access to emergency numbers, their MCFD social worker, police, parents (within the context of their care plan), the Helpline for Children (1-800-663-9122), the RCY, the Ombudsperson, and the local licensing officer at all times



3. STANDARD A — PURPOSE AND FUNCTION OF THE PROGRAM

3.1 Standard A.1: Purpose and Philosophy

Evergreen Community Care maintains a written description of each program's purpose, philosophy, and practices that accurately describes what the program intends to accomplish for children and youth. This complies with Standard A.1 of the MCFD Standards for Staffed Children's Residential Services.

3.2 Written Program Description — Required Content

Each Evergreen Community Care residential program has a written program description that includes:

- Purpose, philosophy, and practices of service delivery
- Population served — age range, gender, presenting needs, and any program specializations
- Admission process and criteria, including pre-placement and post-placement orientation procedures
- Conflict resolution procedure for children, youth, and their families, with assurance there will be no reprisals for making complaints
- Methods of assessment and development of the caregiver service plan
- Service routines and daily programming structure
- Positive behaviour management practices
- Code of ethics governing staff conduct
- Transition planning approach
- Staffing model including ratios and shift structure
- Names and titles of management, supervisory, and direct care staff and the programs for which they are responsible

3.3 Distribution of Program Description

The written program description, or an accessible summary, is provided to:

- Every child or youth placed with Evergreen Community Care
- Parents of children and youth placed with Evergreen Community Care
- All Evergreen Community Care staff members
- Members of the public who request a copy
- MCFD, upon request
- The local licensing officer

3.4 Annual Review

The written statement of purpose is reviewed and updated annually. Changes are communicated to all affected individuals and organizations, including MCFD and the licensing officer.



4. STANDARD B — RIGHTS OF CHILDREN AND YOUTH IN CARE

4.1 Standard B.1: Informing Children and Youth of Their Rights

Evergreen Community Care ensures every child and youth in care understands their rights under the CFCSA and how to enforce them.

Program Practices

- Staff provide children, youth, and their parents with written information about rights in a timely manner, in language children understand, or in alternative formats when required (Braille, audio, video, other languages)
- Staff are knowledgeable about children's rights and can describe how rights information — including how to contact the Representative for Children and Youth (RCY) — is applied in the program

Management Actions

- Written policy and procedure exists for assisting children, youth, and parents with obtaining information about their rights, including how to contact the RCY
- Information is available in other languages and alternative formats including Braille, video, and audio
- Rights information is reviewed with each child at intake and posted in an accessible location within each facility

4.2 Standard B.2: Complaint Resolution

Children and youth are free to express dissatisfaction with services, and their concerns are addressed seriously, fairly, and without delay or reprisal.

Program Practices

- Staff address concerns and complaints in a serious, fair, and timely manner
- Staff provide children and youth with information about the complaint resolution procedure and assist them to understand when and how to involve relevant authorities, including: the child's social worker; the RCY; the Ombudsperson; the Information and Privacy Commissioner; and the local licensing officer

Management Actions — Complaint Resolution Procedure

1. Child or youth raises concern verbally or in writing with any staff member
2. Staff acknowledges the concern respectfully and documents it
3. Program Manager is notified and investigates within 5 business days
4. Written response is provided to the child or youth within 10 business days
5. If unresolved, the concern is escalated to the Executive Director
6. At any stage, the child, youth, or family may contact MCFD, the RCY (1-800-476-3933), or the Ombudsperson without restriction and without fear of reprisal

No child or youth is ever discouraged from making a complaint or from contacting external authorities. Retaliation against a child for making a complaint is a serious matter subject to immediate corrective action and MCFD notification.

4.3 Standard B.3: Privacy and Personal Belongings

Children and youth have reasonable privacy and possession of their personal belongings.



Program Practices

- Staff protect and respect the rights of privacy and confidentiality in accordance with FOIPPA and CFCSA policy
- Staff encourage children and youth to maintain their personal possessions
- Staff apply reasonable and proper grounds for any personal or property search
- Body or strip searches are strictly prohibited under any circumstance

Management Actions

- Each child is able to receive visitors, including family members and friends, in reasonable privacy and comfort, unless there are specific safety considerations documented in the care plan
- Each child is provided with privacy during discussions with a lawyer, the RCY, the Ombudsperson, a Member of the Legislative Assembly, or a Member of Parliament — without exception
- Written policy and procedure exists governing: access to individual service records; sharing of information; use of confidential information for therapeutic, training, or research purposes; prohibition on photographing or identifying children for public purposes without written permission of the MCFD social worker; transfer of records; and storage and disposal of inactive records
- Each child is provided with both secure and accessible storage adequate for their personal possessions
- Written policy exists defining reasonable and proper grounds for personal and property searches; this policy explicitly states that body or strip searches are not permitted



5. STANDARD C — PERSONAL SAFETY OF CHILDREN AND YOUTH

5.1 Standard C.1: Personal Safety and Protection

Children and youth are provided services in a safe environment in which they are protected from harm, or the threat of harm, from self or others.

Program Practices

- Staff screen and monitor all students, volunteers, and visitors to the program
- Staff take action to prevent, or respond to, harmful and threatening circumstances involving children and youth
- Staff ensure water sources — including baths, showers, pools, hot tubs, and wash basins — are at a safe and comfortable temperature. Recommended bathing temperature: 98–100°F (37–38°C). Maximum: 107°F (42°C). Children's skin is more sensitive — when in doubt, err on the side of cooler water and check temperature regularly. (RCR, amended April 2026)
- Staff check the temperature of beverages, including bottles and drinks, to ensure they are a safe and age/developmentally appropriate temperature for each child or youth. (RCR, amended April 2026)
- Staff ensure children and youth have access to: emergency numbers; the child's social worker; police; parents (within the context of the care plan); the Helpline for Children (1-800-663-9122); the RCY; the Ombudsperson; the local licensing officer; and other relevant professionals
- Staff ensure all contact they have with children and youth takes place within the context of the program
- Staff maintain daily logs and carry out wellness checks for every child, every shift
- Staff ensure children and youth receive appropriate gender-affirming care and feel safe in their environment (C.1.1(b), Standard C.1, amended April 2024)

Management Actions — Written Policy Required

- Screening and monitoring of staff, students, volunteers, and visitors
- Ensuring children and youth receive gender-affirming care to promote their safety, well-being, and belonging
- Prevention of and response to harm or threat of harm from self or others
- Ensuring all water sources and liquids are at safe, comfortable, age/developmentally appropriate temperatures, including adding a temperature regulator or scald guard to taps and hot water tanks
- Ensuring children and youth have access to all forms of assistance listed above
- Action to be taken by staff in the event of harmful or threatening circumstances
- Requirement that all contact between children/youth and staff takes place within the context of the program
- Daily logs for each child containing a record of significant aspects of their life, current daily routines, and any changes in routines
- Daily safety and wellness checks on each child and youth within the context of their care plan

5.2 Standard C.2: Reportable Incidents



All information of significance to the safety and well-being of children and youth is promptly reported to the MCFD social worker. Evergreen Community Care maintains written policy and procedure regarding all reportable incidents consistent with legislative, regulatory, and contractual requirements.

Reportable Incidents Requiring Immediate Notification of the Child's Social Worker

The following incidents require immediate notification of the child's social worker so the social worker can plan for the ongoing safety and well-being of the child:

- Death of a child or youth
- Accident or illness requiring medical treatment or hospitalization
- Allegations of abuse, neglect, or mistreatment of a child or youth
- Any displays of self-injurious or high-risk behaviour by a child or youth
- Gestures, threats, or attempts of suicide by a child or youth
- Situations when a child or youth is missing, including any subsequent information obtained about the child or youth while they are missing (see Section 12 — Missing Child/Youth Protocol)
- Situations when a child or youth has observed, been involved in, or been exposed to a high-risk situation or disaster, such as a fire or multiple abuse situation, that may cause emotional trauma or post-traumatic stress
- Any intervention by police or law enforcement authorities with a child or youth
- Use of physical restraint
- Prohibited behaviour management practices, including the use of seclusion/safe rooms
- Unauthorized removal or attempted removal of a child or youth from home, facility, school, or day program
- Marked behaviour changes exhibited by a child or youth
- Suspension of a child or youth from school or day program
- Plans, not previously authorized, for the child or youth to be cared for by another person overnight
- Relocation of a child from one care setting to another
- Any other circumstance affecting the safety or well-being of a child or youth

Additional Reportable Incidents to the Resource Social Worker

- Limitation in the ability of the caregiver to meet the safety and well-being needs of a child or youth
- Limitation in the ability of the caregiver to meet other written caregiver service expectations
- Criminal charge or conviction relevant under the Criminal Records Review Act, of a staff member or other person in the program who has access to a child or youth
- Court-supervised parole or probation of a staff member or other person who has access to a child or youth
- The onset or recurrence of a physical, emotional, or mental condition or substance abuse problem of a contractor or caregiver that could reasonably be expected to impair the caregiver's ability to care for a child

5.3 Standard C.3: Use of Physical Restraint

Physical restraint is used only in extraordinary circumstances to protect the child, youth, or others from immediate physical harm. Evergreen Community Care's approach to restraint fully

complies with MCFD Policy 5.13: Restraint and Seclusion Regarding Children and Youth in Care, and Standard C.3.

Conditions for Use

- Less restrictive means have been considered or attempted and determined to be insufficient in the circumstances, OR
- Physical restraint has been approved as an extraordinary measure by a designated director and is outlined in the child's individual care plan
- Physical restraint involves the least control necessary to ensure immediate safety, appropriate to the child's level of development and condition
- Efforts are made to re-establish communication and end restraint as soon as possible
- The child or youth is NEVER left unattended while under restraint

Required Actions Following Any Use of Physical Restraint

7. Staff who used restraint explain the reasons to the child or youth
8. A debriefing is conducted with the involved child, any youth who witnessed the incident, and staff — in a timely manner
9. The child's or youth's physical and emotional well-being is assessed by staff
10. The incident is documented, incorporating both child/youth and staff perspectives
11. The MCFD social worker is notified immediately
12. Staff who used restraint are provided access to personal debriefing, supervision, risk assessment, and ongoing training and direction

Prohibited Practices — Seclusion

Seclusion or confinement — the containment of a child/youth within any space from which they are not able to exit without the permission of another, or within which their movements may be restricted — is absolutely prohibited. This prohibition does not include the behavioural approach of "time-out" where the child exercises a degree of cooperation and self-control; time-out periods are brief, have a predetermined maximum, and take into consideration the developmental needs of the child.

Management Actions — Written Policy Required

- Required and appropriate training and ongoing supervision of staff in the use of physical restraint
- Procedure for team strategy for safe response
- Safe and least intrusive interventions
- Assessment of child's physical and emotional well-being after restraint
- Documentation requirements
- Reporting requirements, including notification of MCFD social worker
- Debriefing with involved child or youth, child or youth witnesses, the child's social worker, and, where appropriate, the child's parent

6. STANDARD D — QUALITY OF SERVICE EXPERIENCES

6.1 Standard D.1: Service Environment

Children and youth experience an environment in which their spirit, dignity, and individuality are respected.

Program Practices

- Staff listen to and treat children and youth with respect
- Staff encourage and support children and youth to maintain their cultural heritage
- Staff provide Indigenous children and youth with opportunities to connect with and be involved in their Indigenous community
- Staff provide children and youth with opportunities to choose whether or not to participate in any religious instruction or activities
- Staff take steps to prevent discrimination on the basis of gender, sexual orientation, religion, traditions, customs, or language
- Staff provide gender-affirming care that supports children and youth's sense of safety, well-being, and belonging, and respects their gender identity and expression (D.1.13(b), amended April 2024)
- Staff provide children and youth an opportunity to give feedback about the program and the premises
- Staff identify and acknowledge significant events in children's and youth's lives

Management Actions — Written Policy Required

- Listening to and demonstrating respect for children and youth
- Encouraging and supporting children and youth to maintain their cultural heritage
- Providing children and youth of Indigenous heritage opportunities to maintain positive contact, involvement, and participation with their Indigenous community
- Providing opportunities to choose whether or not to participate in religious instruction or activities
- Preventing discrimination on the basis of gender, sexual orientation, religion, race, culture, or language
- Providing gender-affirming care that supports children and youth's gender identity and expression and provides a safe environment
- Providing children and youth with an opportunity to give feedback about the program and premises
- Identifying and acknowledging significant events in the lives of children and youth

6.2 Standard D.2: Family Involvement

Children and youth receive services within the context of the care plan that support the participation of their families and others of significance to them.

Program Practices

- Staff assist children and youth to identify family members and others of significance
- Staff support the participation of children's and youth's family members and others of significance in the program

Management Actions — Written Policy Required

- Providing children and youth with an opportunity to identify family members and others of significance
- Supporting the participation of families and others of significance — including, for Indigenous children and youth, their Indigenous community

Indigenous families and Indigenous communities are jointly responsible for the upbringing and well-being of their children and youth. Indigenous people are involved in the planning and delivery of services to Indigenous families and their children.

6.3 Standard D.3: Maintaining Relationships

Children and youth are encouraged and supported to maintain communication with their families and others of significance, including relationships with their Indigenous community.

Program Practices

- Staff actively pursue opportunities to promote or restore contact between children or youth and their parents, family, and/or others of significance within the context of the care plan
- Staff support children and youth in coping with the impact of either having or not having communication or contact with parents, family, or others of significance

Management Actions — Written Policy Required

- Encouraging and supporting children and youth in maintaining communication or contact with their families and others of significance within the context of the care plan
- Guiding staff in assisting children and youth with coping with the impact of having or not having contact

For Indigenous children and youth: staff guide, encourage, and support connections with extended family, Indigenous community members, the Delegated Aboriginal Agency (DAA) serving their community if applicable, and other related Indigenous organizations or cultural Knowledge Keepers.

6.4 Standard D.4: Community Inclusion

Children and youth receive assistance that supports their inclusion in a variety of community activities.

Program Practices

- Staff provide children and youth with information about community activities
- Staff support the participation of children and youth in a variety of community activities

Management Actions

- Written policy and procedure guides the provision of information about community activities and support for participation

6.5 Standard D.5: Access to Services and Community

Children and youth have reasonable access to community services external to the program and to the local community.

Program Practices

- Staff facilitate children and youth to have reasonable access to services provided in the community
- Staff facilitate children and youth to have reasonable access to the local community

- Staff have access to and are able to use facilities and equipment that children and youth with disabilities need in order to participate in the program and access community services

Management Actions

- Written policy defines the local community in which community services are available
- Written policy supports children and youth having reasonable access to community services
- Evergreen Community Care provides, within available resources, services that children and youth with disabilities need to participate in the program and community

6.6 Standard D.6: Growth and Development

Children and youth are provided with services that promote their growth and development.

Program Practices — Staff ensure children and youth:

- Are encouraged and supported in their educational goals
- Have their physical and mental health needs addressed in a timely fashion; any serious illness or injury is reported to their social worker and others of significance as per the care plan
- Are assisted with acquiring information on preventative health care
- Are provided with a variety of nutritious meals and snacks
- Have opportunities for positive social interaction at mealtimes with staff and other program participants
- Have opportunities to develop and practise skills in self-sufficiency (self-advocacy, interpersonal relationships, self-care, problem-solving, independent living, social skills, cultural and spiritual activities, vocational exploration, community resources, and use of money)
- Are provided gender-affirming care that supports, affirms, and nurtures their gender identity (D.6 amended April 2024)
- Are encouraged to talk about their feelings
- Have opportunities for positive interaction and are encouraged to think for themselves
- Have opportunities for enjoyable leisure and recreation
- Have opportunities to participate in social and recreational activities according to their abilities and interests

Management Actions — Written Policy Required covering:

- Provision of preventative, routine, or emergency medical or dental care
- Obtaining medical attention for symptoms of serious illness or injury
- Obtaining approval for all over-the-counter and prescribed medications and documenting/notifying parents as authorized in care plans
- Advocating for children and youth to be fully informed regarding prescribed medical procedures or treatment
- Obtaining medical information including Personal Health Numbers and consents required for admission
- Ensuring individual service records contain applicable medical and dental information and all signed consent forms
- Ensuring children and youth have adequate, clean, and appropriate clothing for health, comfort, and physical well-being

- Use of tobacco, e-cigarettes, illegal drugs, and alcohol
- Safe storage and use of medications

6.7 Standard D.7: Behaviour Management

Children and youth receive assistance and support in developing constructive and successful social behaviour.

Positive Behaviour Management Practices — Required

Written policy and procedure identifies positive behaviour management practices. These are actions taken by staff intended to guide the behaviour of the child or youth and are based on:

- Praise, encouragement, and structuring the environment for success
- Being reasonable and related to the nature of the behaviour
- Teaching or modelling appropriate ways of behaving
- Responding to the behaviour as soon as possible
- Being motivated by a desire to assist the child or youth to learn responsibility and self-control

Prohibited Behaviour Management Practices — Absolutely Forbidden

The following are expressly prohibited and may never be used under any circumstance:

- Deprivation of a child's or youth's basic rights or needs, including food, clothing, shelter, or bedding
- Physical discipline (spanking, shaking, slapping, hitting, or any form of corporal punishment)
- Degrading actions (humiliation, ridicule)
- Restraint, other than for the immediate physical safety of the child or youth, other children or youth, the caregiver, or others
- Seclusion/safe rooms or confinement (not including appropriate, cooperative time-out)
- Assignment of unreasonable exercise or work that may be excessive or harmful
- Threats of removal from the care setting to manipulate or coerce behaviour
- Arbitrary or unauthorized denial of visits, telephone contact, or correspondence with family members or guardians
- Application of consequences when it is not certain the individual's behaviour warranted them
- Being disciplined by another child who has not been designated as a temporary caregiver
- Coercive behaviour by caregiver(s) or staff regarding their religious or personal beliefs

Management Actions

- Written documentation policy for events leading to a behaviour of concern and subsequent disciplinary action
- Families of children and youth are informed about behaviour expectations and prohibited behaviour management practices
- Consistency and fairness in managing the behaviour of children and youth and in working with involved families

6.8 Standard D.8: Decision Making



Children and youth take part in making decisions affecting their experiences in the program, and can choose from available options.

Program Practices

- Staff provide opportunities for children and youth to have input into decisions affecting their experience in the program
- Staff present available options to children and youth for their consideration
- Staff provide technical and other assistance to children and youth to help them communicate their thoughts and ideas

Management Actions

- Written policy guides children and youth having consultation and input into decisions affecting their service experience
- The presentation of options available for their consideration
- Consideration of both individual and group consultation in assisting children and youth to arrive at decisions
- The provision of technical and other assistance for children and youth to communicate their thoughts and ideas



7. STANDARD E — CHILD AND YOUTH SERVICE PLANNING

7.1 Standard E.1: Eligibility and Intake

Evergreen Community Care, in conjunction with MCFD, has established eligibility criteria, as well as intake and assessment processes.

Written Policy and Procedure — Eligibility Criteria and Intake

- Referral procedures, including the required contact and documentation from the MCFD social worker
- Obtaining client information necessary to provide service, which may include: living arrangements, demographic information, family and cultural information, psychological, social, developmental, behavioural, educational, vocational, legal, and medical information — and ensuring this information is shared with relevant authorized persons
- An assessment process, mutually agreed upon with MCFD, to determine that the service is suited to the needs and best interests of the child or youth and of others receiving service
- Documenting intake decisions and conveying them to referring sources within an agreed-upon timeframe
- Intake and assessment information is sufficient to provide service
- Appropriate consents required for intake are documented
- Transfer of personal belongings occurs where relevant to the service

Written Policy and Procedure — Orientation

Each child and youth, and where appropriate their family, receives an orientation to the program that covers:

- Methods of assessment and development of the caregiver's responsibilities under the plan of care
- Expectations of children and youth
- Behaviour management practices, including prohibited behaviour management practices
- Complaint resolution procedure for children, youth, and their families
- Normal service routines
- Name and title of supervisory and management staff and the program areas for which they are responsible
- Emergency practices and procedures
- Introduction to staff members and other program participants, physical facilities, and equipment

7.2 Standard E.2: Coordination and Integration of Services

Children and youth receive coordinated and integrated services.

Program Practices

- Staff assist children, youth, and involved families with gathering information about the full range of services they are receiving
- Staff assist children and youth with gaining access to relevant staff members in related services
- Staff advise children and youth of consultation that has occurred between caregivers and other service providers

- Staff ensure consistent messages and instruction are provided within the context of the particular service
- Staff coordinate their service with that of other relevant caregivers and service providers and, to the extent possible, directly involve children and youth in this coordination

Management Actions

- Written policy ensures assistance is provided with gathering information about the full range of services being received
- Written policy ensures messages and instruction provided are not contradictory
- Service provision is coordinated and, to the extent possible, integrated with those of other relevant caregivers and service providers

7.3 Standard E.3: Family Inclusion in Service Provision

Families and individuals of significance are encouraged and supported to actively participate in planning for, and providing service to, children and youth.

For Indigenous children and youth, all planning takes into consideration:

- The principle that all Indigenous families and Indigenous communities share responsibility for the upbringing and well-being of Indigenous children
- The importance of the child/youth being able to learn and practise their Indigenous traditions, customs, and language, and belong to their Indigenous community
- The impact of colonization, in particular residential schools, to assess strengths and recognize resiliency when planning and delivering services

Program Practices

- Staff identify family members, others of significance, and appropriate Indigenous communities with children and youth
- Staff support participation of family members, others of significance, and appropriate Indigenous communities
- Staff solicit and consider the views of children and youth, their parents, others of significance, and appropriate Indigenous communities
- Staff record views that have been solicited and inform children and youth of the impact of these views on the services they receive

Management Actions — Written Policy Required covering:

- Providing children and youth with an opportunity to identify family members and others of significance, and for Indigenous children, their Indigenous community
- Supporting participation of families and others of significance, and for Indigenous children, their Indigenous community
- Soliciting the views of children and youth and informing them of the impact of their views on services received
- Opportunities for children and youth, their parents, others of significance, and appropriate Indigenous communities to participate in case conferences, development of caregiver responsibilities under the care plan, and program reviews
- Ensuring all of the above information is recorded in the individual service record for each child or youth

7.4 Standard E.4: Caregiver's Responsibilities Under the Child's Care Plan

Each child or youth is aware of the caregiver's responsibilities under their comprehensive plan of care, which is developed in conjunction with the child or youth, family members, the caregiver, and others of significance.

Program Practices

- Staff carry out the caregiver's responsibilities under the child's care plan
- Staff guide, encourage, and support children and youth's contact and involvement with their Indigenous community by: involving the child/youth, their family, and individuals from their Indigenous community in assessments and development of the care plan; supporting them to learn about and practise their Indigenous traditions, customs, and language; supporting them to belong to their Indigenous community

Management Actions — Caregiver Responsibilities under the Care Plan must include:

- Consultation with and participation of children or youth, parents, and others of significance
- Required cooperation and compliance with court orders relating to the children or youth
- Types of service and intervention required, including specific detail about monitoring of safety and well-being
- Individual goals of the child or youth
- Strengths, aptitudes, and wishes of the child or youth
- Family strengths
- Estimated length of participation in the program
- Anticipated follow-up services
- Documentation of input from children or youth, parents, and others involved

7.5 Standard E.5: Review of Caregiver's Responsibilities

Caregivers ensure timely development and review of their responsibilities under the child's care plan.

Program Practices

- Regular reviews of the caregiver's responsibilities under the child's care plan are conducted and recorded in the individual service record
- Reviews are conducted in a manner respectful of the child or youth concerned and with minimal service disruption to other children or youth
- Reviews are used to effectively update the caregiver's responsibilities under the care plan and specify responsibilities of the caregiver, the child/youth, families, and others of significance
- Children and youth are provided with sufficient time and assistance to adequately prepare for reviews
- Reviews take into account: progress made to date; addressing developmental needs; setting measurable objectives; establishing relevant timelines

Management Actions

- A designated person is responsible for coordinating the development and updating of caregiver responsibilities under the care plan
- Timely review dates are scheduled
- The caregiver's responsibilities are developed with the participation of the MCFD social worker, the child or youth, family, and others of significance



- Children and youth are provided with sufficient time and assistance to prepare for reviews

7.6 Standard E.6: Transitional Planning

Children and youth receive services that assist them in making a successful transition to new circumstances as identified in the caregiver's responsibilities under the care plan.

Program Practices

- Staff assist children and youth in coping successfully with changes in services, involving the children's or youth's family as noted in the care plan
- Staff develop an explicit written transition plan, filed in the individual service record, for preparing and supporting children and youth who are about to make a transition to new living circumstances

Management Actions — Written Policy Required covering:

- Assisting children and youth to cope successfully with changes in services, involving families as noted in care plans
- Early preparation and follow-up contact for children and youth prior to their transition to new living circumstances
- Documentation of the transition plan in the individual service record

7.7 Standard E.7: Child's Individual Service Records

Children have individual records of their relevant history and progress that are accurate, secure, and confidential. For Indigenous children and youth, their Indigenous identity is documented, as well as the specific traditions, customs, and languages of their communities.

Required Contents of the Individual Service Record

Record Category	Required Content
Intake Documentation	Intake information, required documentation, and consents
Indigenous Identity	Indigenous identity and any community-specific traditions, customs, and languages
Legal Documents	Court orders, legal status documentation, guardianship information
Health Information	Medical and dental information, immunization records, signed consent forms
Assessment Information	Assessment records relevant to service delivery
Care Plan	Current caregiver's responsibilities under the child's plan of care and related reviews and reports
Daily Record	Day book for each child including normal daily routines — current and recent past
Complaints	Information on complaints made by the child
Incident Documentation	Documentation on reportable incidents involving the child
Service Termination	Documentation of service termination
Other Information	Any other personal information or documents about the child or youth that the program has in its possession



Management Actions — Written Policy Required covering:

- Supporting children and youth to gain access to their individual service records per FOIPPA and CFCSA
- Explaining to children and youth the purpose and content of their records
- Recording the child's personal opinions and contributions, including any disagreement with information in their records
- The timing and conditions under which records are created, maintained, used, and disclosed
- Reasons for omission from records of any required information
- Who has access to personal information and under what conditions
- Prohibition on the destruction of the individual service record

8. STANDARD F — SHELTER AND EQUIPMENT

8.1 Standard F.1: Buildings, Grounds, and Equipment

Children and youth are provided services in a safe physical environment.

Management Actions

- Written policy and procedure guides monitoring and maintenance of a safe and accessible physical environment
- Training in the safe use of facilities and equipment is provided
- Facilities are maintained in a manner consistent with generally accepted neighbourhood standards
- Evergreen Community Care complies with all relevant provincial acts, regulations, and applicable municipal by-laws

Vehicle Safety Requirements

All vehicles used to transport children and youth must:

- Meet Motor Vehicle Act requirements
- Be maintained in a safe condition
- Include age-appropriate certified child restraint systems
- Have adequate insurance coverage including motor vehicle third-party legal liability coverage of at least \$1,000,000
- Be operated in a safe manner by a person with a valid driver's licence of the appropriate classification

When a child or youth is transported in a motor vehicle, the child or youth is properly secured in a certified restraint system in accordance with the Motor Vehicle Act and Regulations and Canadian Motor Vehicle Safety Standards. Whenever possible, children 12 years of age and under are seated in the back seat.

8.2 Standard F.2: Service Setting

Children and youth receive services in a setting maintained in a manner that demonstrates positive regard for their well-being.

Physical Facility Requirements

Evergreen Community Care ensures each residential facility provides:

- Adequate heating and ventilation in all facilities
- Furniture that is safe and adequate to meet the needs of children and youth
- Informal living space accessible to friends and family, conducive to socializing, group activities, visiting, privacy, recreation, and play
- A dining area sufficient to permit children, youth, and staff to eat meals together
- Cooking areas equipped for safe, sanitary preparation of meals that allows for the participation of children and youth
- Toilet facilities located within reasonable access from bedrooms, designed and appropriately equipped for the needs of the children and youth living in the residence
- At least one opening window in each room commonly used for sleeping
- Private workspace for staff conducive to effective performance, including space for administrative duties, privacy for counselling, and interviewing



- Adequate space for secure storage of children's, youth's, and staff members' personal effects
- A variety of media such as print, television, and radio for the entertainment of children and youth

Management Actions

- Written policy incorporates the views of children and youth regarding the service setting
- Written policy guides establishing and maintaining the service setting in a manner that conveys positive regard for children and youth

8.3 Standard F.3: Individual Accommodation

Children and youth are provided with sufficient living space to meet their needs in terms of privacy, safety, and well-being.

Program Practices

- Children and youth have reasonable choice in arranging and decorating their personal space
- Children and youth are assisted in selecting options to keep valued possessions safe and secure
- Children and youth have appropriate space in which to study, do homework, or pursue hobbies

Management Actions — Written Policy Required covering:

- Organizing the residence so individual children and youth are provided with privacy, safety, and well-being within a space identified as their own
- Permitting children and youth to individualize their personal space
- Practice with respect to children and youth locking bedrooms or bathrooms, balancing safety and security with freedom and privacy, having regard to age and understanding
- Providing appropriate space in which each child or youth can study, do homework, or pursue hobbies
- Providing adequate storage for belongings and security for valuables
- Documentation of reasons in cases where adequate storage is not provided

8.4 Standard F.4: Safety and Emergency Practices

Children and youth receive services in which all reasonable steps are taken to keep them safe from the risk of fire and other hazards.

Fire Safety Requirements

Evergreen Community Care ensures the following fire safety precautions are in place and maintained:

- A fire safety/emergency plan is prepared and posted in a conspicuous area on each floor
- Fire drills are practised monthly and immediately following the placement of a child or youth
- Emergency numbers are clearly posted beside each telephone
- All fire extinguishers are maintained in working order and are readily available
- Maximum storage of flammable and combustible liquids in the building does not exceed 10 litres



- All flammable and combustible liquids are stored in ULC-approved containers in areas not readily accessible to children and youth
- All waste receptacles are of non-combustible material
- At least two means of exit to ground level are provided from each floor on which a sleeping room is located
- Fire escape routes are kept clear, well-marked, and safe
- Interconnected smoke alarms are installed in each bedroom and in each corridor leading to a bedroom
- Emergency lighting is installed that will automatically illuminate corridors and stairs for a minimum of 30 minutes in the event of a power failure
- An approved sprinkler system is installed and includes routine testing (exemptions apply per the Community Care Facility Act licensing provisions)
- The wall between the residence and an adjoining garage provides at least a 45-minute fire separation
- Annual fire safety inspections are conducted by the local fire authority (or a notarized Letter of Assurance in the format of Appendix 4 of the MCFD Standards, for settings of maximum 6 children where a local fire authority is not available)

Additional Safety Requirements

- Medication and all poisonous and hazardous materials are stored in a secure place, inaccessible to children and youth
- A first-aid kit is readily available to program staff
- The service setting is kept free of any harmful materials and objects not essential to the program's operation
- Firearms are not kept in or about the service setting, unless authorized in writing by MCFD; where authorized, all firearms are inaccessible to children and youth and stored in accordance with federal legislation

9. STANDARD G — HUMAN RESOURCES

9.1 Standard G.1: Staff Recruitment, Selection, and Screening

The program has thorough selection and screening procedures to ensure the utilization of suitably qualified staff, volunteers, and students.

Required Screening Process — Management Actions

Written policy and procedure governs the selection of all staff, volunteers, students, regular visitors, and others who will have unsupervised contact with children and youth. The process includes:

- Adherence to all relevant legislation including the Criminal Records Review Act and the Community Care and Assisted Living Act
- A personal interview
- Documentation of a criminal record check, including Vulnerable Sector Check, prior to any unsupervised contact with children or youth
- Personal and employment references
- Medical reference
- Assessment of the information gathered against the applicant's capacity to provide for the safety and well-being of children and youth
- No candidate may begin work with children or youth, even on a supervised basis, until a clear Criminal Record Check has been received and verified

Personnel Records — Required Contents

Personnel Record Item	Required
Criminal record check documentation	Yes
Personal and employment references	Yes
Relevant medical history	Yes
Application for employment and/or résumé	Yes
Verification of educational and professional qualifications	Yes
Results of employment interview and reasons for hiring	Yes
Copy of letter of employment and acceptance	Yes
Written acknowledgement of personnel orientation	Yes
Written performance evaluations	Yes
Documentation of any discipline measures	Yes
Dates of commencement and termination of employment	Yes
Recommendations concerning rehiring within the program	Yes
Employee interview upon termination of employment	Yes
Copies of certifications (First Aid, driver's licence, etc.)	If required by role

All personnel records are kept in a secure and retrievable form, accessible upon request by MCFD.



Job Descriptions

Evergreen Community Care maintains current written job descriptions for each regular and relief staff position, outlining required duties and responsibilities, qualifications, education, training, experience, and lines of accountability.

Youth Input on Staff Qualities

Children and youth are provided with an opportunity to express their opinions about those qualities they find helpful or unhelpful in staff, volunteers, and students. This input is documented and considered in personnel decisions.

9.2 Standard G.2: Staff Complement

The program has enough staff to achieve its stated purpose and to meet the needs of the children and youth it serves.

Staffing Ratios

Facility Type	Daytime Ratio	Overnight Ratio	Notes
Group Home (up to 4 children)	1:4 minimum, or as per license	1 awake staff	Per RCR requirements
Group Home (5–6 children)	2 staff minimum	1 awake staff	Per RCR requirements
Specialized / High-Needs Care	As specified in Care Plan	1:1 or as required	Determined by MCFD and care plan
Emergency / Short-Term Placement	1:2 minimum	1 awake staff	

- Evergreen Community Care monitors and documents any discrepancy between required and actual staffing levels
- A written plan exists to provide additional emergency staff when required
- Staff complement includes consideration of age, gender, cultural background, and personal needs of the children and youth being served

9.3 Standard G.3: Training and Staff Development

Program staff are provided with ongoing training to meet job requirements.

Required Orientation for All New Staff

All new staff receive written orientation information covering:

- Caregiver's philosophy and services
- Service description, goals, and procedures
- Job requirements and duties, including the caregiver's responsibilities under the children's comprehensive plans of care, and any service needs specific to each child for whom the staff member is responsible
- Safety requirements, emergency procedures, and reportable incidents
- Communication and confidentiality
- Code of ethics
- The MCFD Standards for Staffed Children's Residential Services and their application



- Gender-affirming care principles and mandatory training requirement (G.3.2(b), amended April 2024)

All new staff acknowledge in writing that they have read and understood the above information. This signed acknowledgement is retained in the personnel file.

Required Training Matrix

Training	Orientation Deadline	Annual Renewal
First Aid / CPR (Level C)	Before first unsupervised shift	Per certification cycle
NVCI or equivalent crisis intervention incl. safe use of restraint	Within 90 days	Annual recertification
Trauma-Informed Practice	Within 60 days	Annual refresher
Indigenous Cultural Safety and Humility	Within 60 days	Annual refresher
Gender-Affirming Care (mandatory — Standard G.3.2(b))	Within 60 days	Annual refresher
LGBTQ2S+ Inclusion	Within 60 days	Annual refresher
Duty to Report (CFCSA s.14)	Within 30 days	Annual refresher
Privacy and FOIPPA	Within 30 days	Annual refresher
Medication Administration (if applicable)	Within 60 days	Annual competency check
Abuse Prevention and Recognition	Within 60 days	Annual refresher
Missing Child/Youth Protocol (Appendix 1)	Within 30 days	Annual refresher
WHMIS / Workplace Safety	Before first shift	As required

9.4 Standard G.4: Staff Support and Supervision

Program staff receive the support and supervision needed to perform their job requirements.

Program Practices — Regular Supervision Covers

- Responses to children and youth, including those children and youth for whom the staff member has direct responsibility
- Methods of working with children and youth
- Providing gender-affirming care to children and youth (G.4.3(c), amended April 2024)
- Promoting the safety and well-being of children and youth
- Degree of personal involvement
- Individual feelings and concerns relating to job requirements
- Staff development and training

Communication Mechanisms

- Staff meetings
- Briefing sessions
- Shift change meetings
- Daily logs and other communications
- Staff team days



Management Actions — Written Policy Required covering:

- Providing each staff member with a written job description
- Establishing a probationary period for new staff
- Providing staff with regular supervision and support to monitor and enhance job performance
- Providing regular, written performance evaluations
- Personnel disciplinary procedures consistent with the HR Policy Binder (ECC-HR-POL-2026-001)
- Staffing ratios and work schedules relative to service objectives and supervision of children and youth, including a written plan to provide additional emergency staff when required
- Job classifications, hours of work, remuneration, and employee benefits for all positions
- Promoting a work environment free from harassment
- Establishing and maintaining staff grievance procedures
- Regular review and implementation of measures to promote health and safety in the working environment

10. STANDARD H — ORGANIZATION AND MANAGEMENT

10.1 Standard H.1: Administration and Management

The program is administered in an ethical and accountable manner consistent with its stated purpose.

Legislative and Contractual Compliance

- Evergreen Community Care complies with all applicable Acts, regulations, and policies of the Government of Canada, the Province of British Columbia, and all applicable municipal by-laws
- Complies with all contracted service requirements, including MCFD Service Standards
- Provides all documentation and records as required by MCFD
- Where required, maintains "good standing" with the Registrar of Companies and Societies and with professional, employer, and other associations

Governance and Accountability

- The Executive Director is responsible and accountable for the organization's policy, procedure, and activities
- A designating signing officer is identified
- A current, written policy and procedure manual relating to service delivery is maintained (this document)
- Adherence to all policy and procedure is ensured
- Policy and procedure is reviewed and revised as necessary
- Changes to policy and procedure, including additions, revisions, and deletions, are communicated to all affected individuals, agencies, MCFD, and any regulating bodies
- A current organizational chart clearly identifies lines of authority, responsibility, and accountability within the organization; it is shared with MCFD and updated upon any change
- A list including names and business addresses of managerial staff occupying positions within the organization is maintained and shared with MCFD

Financial Management Requirements

- An accounting system consistent with generally accepted accounting principles is maintained
- Written policy and procedure governs: receipt and disbursement of funds, goods, and services; regular review and revision of budgets; purchasing, inventory control, investments, and conflicts of interest
- An annual budget is produced
- Monthly financial statements accurately record all income and expenditures
- An annual financial report is produced as required by MCFD
- Financial records are stored in a retrievable form
- Written policies and procedures govern fund-raising activities in accord with applicable law

Insurance Requirements

- Property, vehicle, and liability insurance that is adequate and appropriate for the program's operation is obtained and maintained



- Insurance policies are reviewed annually to ensure adequate and appropriate levels of coverage
- All vehicles used to transport children and youth are maintained in good running order and comply with the Motor Vehicle Act
- Insurance policies include provision for casual support staff, student placements, and volunteers
- Written policy and procedure outlines safety inspections, accident prevention practices, and emergency procedures

Code of Ethics

Evergreen Community Care adheres to a documented code of ethics that includes reference to:

- Conflicts of interest
- Workplace behaviour and relationships
- Gifts and complimentary items
- Religious and cultural influence
- Remuneration
- Confidentiality

10.2 Standard H.2: Review and Evaluation

Evergreen Community Care promotes quality of service by ensuring ongoing program review and evaluation.

Annual Program Review

The caregiver annually reviews its program to determine:

- Need for and relevance of services
- Attainment of goals related to the caregiver's responsibilities under each child's or youth's plan of care
- Attainment of the overall goals and objectives for the program
- Human and material resource management
- Current relevance of policy and procedure

Evaluation Process

- Written policy ensures participation of children, youth, and their families, staff members, and other relevant parties in the evaluation of caregiver responsibilities under children's or youth's plans of care, as well as overall program goals and objectives
- Evaluation of caregiver responsibilities and overall program goals is guided by the MCFD Standards for Staffed Children's Residential Services
- Evaluation results are used to improve outcomes for children and youth
- MCFD is provided with a copy of the complete evaluation report, including a summary of significant findings and recommended service changes
- A summary of the evaluation, including significant findings and recommended service changes, is shared with program staff, children and youth, and other relevant parties

11. INDIGENOUS RECONCILIATION FRAMEWORK AND CULTURAL SAFETY

11.1 Truth and Reconciliation Commitment

Evergreen Community Care acknowledges the profound and ongoing impact of colonization and residential schools on Indigenous children, families, and communities. We are guided by the Truth and Reconciliation Commission's Calls to Action, the Declaration on the Rights of Indigenous Peoples Act (DRIPA), and UNDRIP in all aspects of our service delivery.

11.2 Indigenous Placement Principles — CFCSA s.70

In accordance with section 70 of the CFCSA and DRIPA, placement preference follows this order:

13. Indigenous family members of the child
14. Extended family or persons nominated by the child's family
15. A family approved by the child's Indigenous community
16. Another Indigenous family
17. A non-Indigenous family that will support the child's cultural identity

11.3 Cultural Plans

A Cultural Plan must be developed for every Indigenous child or youth within 30 days of placement, in collaboration with the child, their family, and their Nation. The Cultural Plan documents:

- The child's specific cultural background, language, and community
- Goals for maintaining and strengthening cultural connection
- Activities, ceremonies, and community participation opportunities
- Responsibilities of Evergreen Community Care, the child's MCFD social worker, and the Nation in supporting cultural goals
- Connection to Elders, cultural mentors, the Delegated Aboriginal Agency (DAA), and Friendship Centres

11.4 Nation and Community Notification

- Evergreen Community Care notifies the relevant Indigenous community or Nation prior to or upon placement of an Indigenous child, consistent with any agreement in place between the Director and the Indigenous community under s.92.1 of the CFCSA
- Band social workers and community representatives are included in care team meetings as appropriate

11.5 Cultural Practices in the Home

- Smudging, ceremony, drumming, traditional foods, and other cultural practices are accommodated and facilitated within the home
- Sacred items are treated with respect; youth are supported in accessing traditional medicines and healing practices
- Staff complete annual Indigenous Cultural Safety and Humility training



12. GENDER-AFFIRMING CARE POLICY

12.1 Legal Basis

Gender-affirming care is defined in the MCFD Standards for Staffed Children's Residential Services (April 2024 revision) as: the processes through which caregivers, staff, services, and organizations care for and support an individual, while recognizing and acknowledging their gender identity and expression to promote their safety, well-being, and belonging.

This policy reflects mandatory standards C.1.1(b), D.1.5(b), D.1.13(b), D.6 (g), G.3.2(b), and G.4.3(c) as amended April 2024.

12.2 Staff Obligations

- All staff use each child's or youth's stated name and pronouns consistently
- All staff complete mandatory gender-affirming care training within 60 days of hire and annually thereafter
- Staff provide gender-affirming care that supports, affirms, and nurtures children and youth's gender identity and expression
- Staff take active steps to ensure the physical environment is safe and affirming for children and youth of all gender identities

12.3 Confidentiality

- A child's or youth's gender identity or expression is treated as confidential personal information
- It is not shared with family, MCFD, or others without the child's or youth's informed consent, except where there is a documented safety reason

12.4 Environment and Supports

- Bedroom and washroom arrangements reflect a child's or youth's gender identity wherever operationally feasible and with the child's input
- Youth are provided access to gender-affirming clothing, personal care items, and mental health resources as part of their normal care
- Zero tolerance for harassment or discrimination based on gender identity or expression, by peers, staff, or visitors

13. MISSING CHILD / YOUTH PROTOCOL (APPENDIX 1)

13.1 Definitions

A missing child or youth is any child/youth in care, including respite care, whose whereabouts are unknown and who cannot be found after taking the same actions as a prudent and responsible parent.

Provincial Centralized Screening (PCS): 1-800-663-9122 — provides 24/7 child protection service across the province.

13.2 When a Child's or Youth's Whereabouts Are Unknown

Before contacting PCS or the child's social worker, staff take the same actions a prudent and responsible parent would, considering the child's age, developmental needs, circumstances, and other contextual factors:

- Actively try to contact the child or youth directly via their cell phone or other personal device
- Actively seek and contact anyone who may know the child's whereabouts — friends, family, persons they are in a relationship with, members of the child's Indigenous community, coaches, teachers, community professionals
- Conduct a thorough search of the premises including the child's room, the home, and outdoor areas
- Identify and check likely locations in the community

If there are no immediate health or safety concerns, contact the child's worker or PCS to determine if police/RCMP involvement is needed. If you believe the child's health or safety is in immediate danger at any time, contact police/RCMP first, then contact the child's worker or call PCS (option 1 — immediate queue).

13.3 When the Child Is Confirmed Missing

If the above actions do not result in locating the child, the child is considered missing. Staff must:

18. Follow any specific missing person plan the child's worker has provided
19. If no plan exists, contact the child's worker or PCS Immediate Queue
20. If there is reason to believe the child's health or safety is in immediate danger, call police/RCMP
21. Once contacted, the child's worker or PCS will direct specific next steps

There is no requirement to wait a specific amount of time before reporting a child missing to the child's worker or PCS.

13.4 Information to Provide When Reporting

- Whether it is out of character for the child to be missing
- Physical or mental health concerns that could increase risk of harm
- Whether the child was emotionally distraught when last seen, including any history of suicidal ideation
- Known substance use
- Weather conditions and appropriateness of the child's clothing for conditions



- Any concerns that the disappearance may result from suspicious or dangerous circumstances or violence

Additional Information Required When Reporting to Police/RCMP

- Full name (including aliases), date of birth, and gender
- Address and cell phone number
- Time and place last seen, by whom, and any information about last contact
- Current photo and physical description: ethnicity, height, weight, hair colour/length, eye colour, distinguishing characteristics, last known clothing, and items in possession
- Names, addresses, and phone numbers of anyone who may know the child's whereabouts
- Name of child's school and teacher if applicable
- Factors posing immediate risk (exploitation, gang affiliation, unsafe locations)
- Clear instructions that any information made public must NOT indicate the child is in care under the CFCSA

13.5 When the Child Is No Longer Missing

- The child's worker ensures all parties are notified when the child is no longer missing
- Staff document actions taken and efforts made at the earliest opportunity
- A welfare check and debrief are conducted with the child upon their safe return
- The child's care plan and safety planning are reviewed following any missing person incident



14. POLICY REVIEW AND APPROVAL

14.1 Review Schedule

- Annually each July
- Upon amendment to the MCFD Standards, RCR, CFCSA, or CCALA
- Upon recommendation arising from a licensing inspection, MCFD monitoring visit, or critical incident review

14.2 Approval Authority

Policy Owner	Executive Director, Evergreen Community Care
Approved By	Board of Directors / Executive Director
MCFD Liaison	Regional MCFD Contract Manager / Licensing Officer

14.3 Version History

Version	Date	Summary
1.0	January 2025	Initial policy manual created
2.0	July 1, 2026	Comprehensive revision — manual rebuilt against all 8 standard categories (A–H) of the MCFD Standards for Staffed Children's Residential Services (revised to April 2026), incorporating gender-affirming care requirements (April 2024), water/beverage temperature requirements (April 2026), missing child protocol updates (May 2025), and updated Indigenous service planning requirements



4. ORGANIZATIONAL MISSION, VALUES, AND PROGRAM PHILOSOPHY

Legislative Reference: MCFD Standard A.1 — Written Program Description; CFCSA s.2 (Guiding Principles); CFCSA s.4 (Best Interests of the Child)

4.1 Mission Statement

Evergreen Community Care Inc. provides safe, trauma-informed, rights-based residential care to children and youth placed by the Ministry of Children and Family Development. We are committed to creating a home — not an institution — where every young person experiences stability, belonging, healing, and the genuine support of consistent, caring adults who believe in their capacity to grow.

4.2 Vision

Every child and youth in our care knows they are valued, connected, and capable. They leave our programs with stronger roots and wider horizons than when they arrived.

4.3 Core Values

These are not aspirational statements. They are the operating principles by which Evergreen Community Care hires, trains, supervises, and holds itself accountable:

Value	What This Looks Like in Practice
Children First, Always	Every operational decision — scheduling, staffing, programming, purchasing — is evaluated first through the lens of its impact on children and youth. No operational convenience justifies compromising a young person's safety, rights, or wellbeing.
Safety is the Floor, Not the Ceiling	Physical and psychological safety is the non-negotiable foundation. We do not celebrate meeting minimum safety requirements. We aim higher. An environment that is merely safe enough is not good enough.
Relationships are the Intervention	Research and experience both show that the most powerful factor in a child's healing is a consistent, caring, honest relationship with a trusted adult. Our staffing, scheduling, and supervision structures are built to protect and nurture those relationships.
Trauma is the Lens, Not the Label	Every young person in our care has experienced adversity. We do not use trauma as a category or a shortcut to lower expectations. We use it as a lens for understanding — so that we respond to the need underneath the behaviour, not just the behaviour itself.



Cultural Identity is Non-Negotiable	A child cannot thrive in a setting that treats their cultural identity as a problem to be managed or a preference to be accommodated. For Indigenous children especially, cultural connection is not supplementary programming — it is central to identity, healing, and self-determination. We actively facilitate it.
Voice Belongs to the Young Person	Children and youth in care have had too many decisions made about them without them. At Evergreen Community Care, youth voice is not a courtesy extended when convenient. It shapes care plans, daily routines, programming choices, and our organizational learning.
Accountability Without Blame	When things go wrong — and in residential care, they sometimes will — we investigate honestly, fix what needs fixing, and learn from what happened. We do not hide incidents, minimize errors, or protect organizational reputation at the expense of a child's safety or dignity.
Excellence is a Daily Standard	Good enough is not our target. We recruit for character and competence, provide real training and supervision, hold each other to professional standards, and continuously improve based on evidence, feedback, and reflection.

4.4 Program Philosophy: Therapeutic Residential Care

Evergreen Community Care's residential programs operate from a clearly articulated philosophy of care. Every staff member, supervisor, and manager is expected to understand and actively apply this philosophy in their daily practice — not just be able to recite it.

The Home, Not the Institution

Residential care at Evergreen Community Care looks and feels like a home, not a facility. Meals are cooked and eaten together. Birthdays are celebrated. Spaces are personalized. Youth have privacy, possessions, and a say in how the household runs. The physical environment, the daily routine, and the relational culture all communicate: you belong here.

Consistent Adults as the Primary Vehicle of Change

No program design, treatment modality, or documentation system is more therapeutic than a consistent, caring relationship between a young person and a staff member who genuinely believes in them. We staff for relationship continuity. We supervise for reflective practice. We build teams where support workers feel known and valued — because workers who feel cared for provide better care.

Graduated Autonomy and Capacity Building

Our programs are designed to increase, not maintain, each young person's independence. Every placement includes deliberate attention to life skills, community connections, educational engagement, and the development of a support network that will exist after the formal care relationship ends. We are preparing young people for life — not just managing their placement.



Honest, Compassionate Limit-Setting

We set clear, consistent expectations because structure and predictability are themselves therapeutic for children who have experienced chaos and instability. We enforce limits calmly, non-punitively, and relationally — explaining the reason, maintaining dignity, and repairing the relationship after any difficult moment. We do not mistake compliance for safety or submission for wellbeing.

Continuous Learning and Honest Self-Assessment

Residential care is complex, high-stakes, and demanding. We do not claim to have all the answers. We commit to asking the right questions regularly: Are the children in our care safer and better supported than they were last month? What are we learning from incidents, complaints, and feedback? What needs to change? This honest self-assessment is built into our quality assurance cycle, our supervision practice, and our leadership culture.

4.5 The Reasonable and Prudent Parent Standard

The CFCSA and BC Residential Care Regulation require that residential care providers apply the standard of the “reasonable and prudent parent” in decisions about the children in their care. This means:

- We make decisions about activities, risks, and opportunities the way a thoughtful, involved parent would — not with institutional caution that prevents young people from having ordinary experiences
- We do not restrict youth from activities simply because those activities carry some ordinary risk. A child who cannot attend a birthday party, try a new sport, or make a mistake and experience its natural consequence is not being protected — they are being over-managed
- We document our decision-making for significant choices and are prepared to explain our reasoning to MCFD, a licensing officer, or a court
- The prudent parent standard does not conflict with youth safety — it contextualizes it. A prudent parent manages real risks while allowing genuine lived experience

4.6 Our Commitment to MCFD and Licensing Partners

Evergreen Community Care enters every MCFD contracted service relationship with a clear and genuine commitment to transparency, compliance, and partnership. We do not treat MCFD monitoring or licensing inspections as obstacles. They are external verification that the care we deliver matches the care we commit to on paper. We welcome that accountability.

When we identify gaps in our own practice — through incident reviews, staff feedback, youth complaints, or our own quality assurance audits — we raise them proactively, not defensively. We treat every gap as a system to fix, not an individual to blame.

PART III — OPERATIONAL PRACTICE STANDARDS



16. ADMISSION, INTAKE, AND ORIENTATION

MCFD Standards A.1, E.1; Residential Care Regulation s.7–10; CFCSA s.70

16.1 Pre-Admission Assessment

Before accepting any placement, the Program Manager reviews all available documentation and confirms:

- The referral comes from or is authorized by MCFD or a Delegated Aboriginal Agency (DAA)
- The facility has current licensed capacity for the placement
- The referred youth's assessed needs are within the program's current capacity to meet safely
- The placement does not compromise the safety or wellbeing of youth currently in the home
- Any environmental modifications required by the youth's risk profile can be completed before arrival

For emergency placements where full information is not yet available, the Program Manager accepts the placement based on available information and initiates an urgent ISP review within 48 hours.

16.2 Day-of-Admission Requirements

Task	Timeline	Responsible
Youth welcomed; bedroom assigned and shown; belongings inventoried	Day 1	Admitting staff
Rights information reviewed verbally; Appendix C handout provided	Within 4 hours of arrival	Admitting staff
Emergency contacts and Advocate (1-800-476-3933) shown and explained	Day 1	Admitting staff
All medications received, inventoried, locked; MAR initiated	Day 1	Medication-authorized staff
Hot water temperature at all taps confirmed not exceeding 49°C (RCR April 2026)	Day 1	Program Manager
Current court orders and contact restrictions reviewed	Day 1	Program Manager



Environmental risk assessment reviewed for this youth's specific risk profile	Within 24 hours	Program Manager
Indigenous status confirmed; Cultural Plan meeting scheduled within 30 days	Day 1	Program Manager
Preliminary ISP / safety plan documented and in use	Within 48 hours	Program Manager
Full ISP meeting scheduled with MCFD social worker	Within 14 days	Program Manager
Orientation completed using Appendix B	Within 48 hours	Designated staff

16.3 Youth Orientation

Every youth receives a structured orientation within 48 hours of arrival, delivered at their pace using the New Youth Orientation Guide (Appendix B). Orientation covers: tour of the home; daily routine; their rights and how to exercise them; how to make a complaint; how to contact the Advocate; emergency procedures; house expectations; and introduction to all staff currently on duty.

17. INDIVIDUAL SERVICE RECORDS AND DOCUMENTATION

Residential Care Regulation s.10–16; MCFD Standards E.7, H.1; FOIPPA; CFCSA s.15

17.1 Required Records for Every Youth

Record Type	Content and Minimum Retention
Admission documentation	Referral, consent forms, legal status, court orders, emergency contacts, intake assessment; 7 years post-discharge
Individual Service Plan (ISP)	Current and all prior versions including Behaviour Support Plan; 7 years post-discharge
Medication Administration Record (MAR)	Complete record for every medication, every month; 10 years
Shift log / daily notes	Factual contemporaneous record of significant events each shift for each youth; 7 years
Health care log	All appointments, diagnoses, treatments, provider communications; 7 years
Education log	School enrolment, attendance, report cards, meeting records; 7 years
Critical Incident Reports	All categories; permanently retained (allegations of abuse: never destroyed)
Family contact log	Date, type, duration, persons, youth response, any concerns; 7 years
Court orders and legal documents	All orders affecting the placement; custody documentation; permanently retained
Cultural Plan (Indigenous youth)	Current and all prior versions; 7 years post-discharge
Transition plan and discharge summary	Completed at each transition; 7 years post-discharge
Financial records (allowances, trust funds)	All transactions; 7 years

17.2 Shift Log Standards

The shift log is a legal document and the continuous narrative record of a young person's life in the program. Every shift log entry is:

- Accurate: it records what was observed and said, not what was assumed or inferred



- Objective: language is factual and non-judgmental; specific and observable rather than interpretive
- Contemporaneous: completed before staff leave the facility at the end of their shift
- Complete: significant events are not omitted because they reflect poorly on the program or a staff member
- Professional: no derogatory language, clinical labels not supported by formal diagnosis, or personal commentary

⚠ Falsified, altered, or materially incomplete documentation is a serious professional and potentially legal matter. Staff are required to document accurately, including when events reflect a mistake. Omitted documentation is treated as non-occurrence in any MCFD, licensing, or legal review.

17.3 Confidentiality and Access

- ISRs are stored in locked files; electronic records in access-controlled, password-protected systems
- Access is limited to: authorized Evergreen Community Care staff with current care need; MCFD in their role managing the child's care; Health Authority Licensing Officers; the youth themselves (age-appropriate, supported access)
- All requests for records from family members, police, courts, or external organizations are referred to the Program Manager; no records are released without legal authority or executive authorization
- A youth may request access to their own ISR; facilitated within 30 days per FOIPPA

18. CARE PLANNING AND INDIVIDUAL SERVICE PLAN (ISP) PROCESS

MCFD Standards E.4, E.5; CFCSA s.70(1)(b); MCFD Child Welfare Policy Chapter 4

18.1 ISP Development — Non-Negotiable Principles

The ISP is the central document guiding every aspect of a youth's care. It is developed with the youth — not for them. The following principles are non-negotiable:

- The youth's own voice, goals, and preferences are placed at the centre of the ISP — not listed at the end as a formality
- The ISP is written in language the youth can understand; they receive a copy in a format accessible to them
- Every ISP goal has a named responsible party and a review mechanism; goals without accountability are not goals
- The ISP is a living document that evolves with the youth — not a compliance document produced at intake and filed

18.2 Required ISP Components

Component	Content Required
Strengths and identity	Youth's strengths, interests, cultural identity, and personal goals — developed with the youth's active input
Goals by domain	Safety, health, education, emotional wellbeing, relationships, cultural connection, life skills, and transition
Behaviour Support Plan (BSP)	For youth with identified behaviour concerns: triggers, de-escalation strategies, approved interventions, replacement behaviours
Safety plan	Identified risks; specific safety strategies; supervision level; environmental modifications required
Medication plan	All current medications, dosages, prescribers, administration schedules, and any consent or refusal documentation
Health alerts	Allergies, diagnosed conditions, emergency procedures specific to this youth
Education plan	Current school/program; attendance expectations; support needs; IEP status
Family and contact plan	Authorized contacts; visit and call schedule; any restrictions and documented reason for each
Cultural plan	For Indigenous youth: Nation, cultural activities, community connections, DAA involvement, traditional practices



Gender affirmation plan	Where applicable: preferred name, pronouns, specific affirming environment requirements
Permanency goal	The long-term goal as established by MCFD; transition planning milestones
Youth signature or acknowledgment	Youth's signed acknowledgment or documented reason if youth declined to sign

18.3 ISP Review Timelines

Review Trigger	Timeline
Initial ISP (new placement)	Interim plan within 48 hours; full ISP meeting within 14 days
Regular scheduled review	Minimum every 90 days (quarterly)
Following any critical incident	Within 5 business days
Significant behaviour change or health event	Within 5 business days
Approaching transition (discharge or placement change)	Begin transition planning 60 days before anticipated change; update monthly
Youth requests a review	Facilitated within 10 business days of the request



19. HEALTH CARE MANAGEMENT

MCFD Standards D.6, F.1; Residential Care Regulation s.36–45

19.1 Health Care Responsibilities

Evergreen Community Care holds delegated authority to consent to routine health care for children and youth in its care, within the limits of its MCFD service agreement and applicable legislation. Decisions beyond routine care require MCFD social worker approval before implementation, unless an emergency makes prior approval impossible.

19.2 Health Care Access Standards

Care Type	Standard
Family physician	Every youth is established with a GP or NP within 30 days of placement; walk-in clinic used pending establishment
Emergency medical care	911 is called immediately for any medical emergency; a staff member accompanies the youth; MCFD social worker notified within 1 hour
Dental care	Annual dental exam arranged; dental pain treated promptly without waiting for a scheduled appointment
Mental health care	Youth with identified mental health needs are connected with appropriate services within 30 days of identification; MCFD social worker is partner in this
Vision care	Vision needs identified and corrective aids obtained; annual eye exam where indicated
Specialist care	Referrals tracked; appointments attended; reports filed and communicated to the care team and MCFD
Immunizations	Records reviewed at admission; any gaps addressed with the physician and MCFD
Reproductive and sexual health	Non-judgmental access to sexual health services facilitated; consistent with the youth's age and care plan

19.3 Water and Beverage Temperature Safety

As required by the Residential Care Regulation (amended April 1, 2026):

- Hot water at all taps, baths, and showers does not exceed 49°C (120°F); thermostatic mixing valves are installed where required



- All beverages provided to youth — including bottles, heated drinks, and warm foods — are checked for safe, age-appropriate temperature before being given to the youth
- Recommended bathing water temperature: 37–38°C; maximum 42°C; staff check with wrist, not hand



20. MEDICATION ADMINISTRATION PROGRAM

Residential Care Regulation s.46–52; MCFD Standards D.6; HR Policy Binder s.11C

20.1 Authorization

Only staff who hold current Medication Administration authorization — confirmed by written competency assessment in their personnel file — administer any medication to any youth. Authorization is renewed annually. Staff with lapsed authorization are removed from medication duties until reauthorized.

20.2 Medication Storage

Medication Type	Storage Requirement
All prescriptions	Locked medication cabinet; accessible only to authorized staff
Schedule I–III controlled substances	Double-locked storage; counted and reconciled by both outgoing and incoming staff at every shift change
Refrigerated medications	Locked refrigerator section; temperature logged daily
Emergency medications (EpiPen, Diastat, puffer)	Clearly labeled, immediately accessible location known to all on-shift staff; confirmed at every shift change
Expired or discontinued medications	Removed from active cabinet immediately; returned through BC Medications Return program; never placed in general waste

20.3 The Five Rights — Required Before Every Administration

Every medication administration follows the Five Rights, verified before each dose:

- Right youth: confirm the youth's identity before preparing any medication
- Right medication: confirm the name and strength against the current MAR
- Right dose: confirm the prescribed dose; do not assume a historical dose is still current
- Right route: confirm the method of administration (oral, topical, inhaled, etc.)
- Right time: confirm the prescribed time window; document the actual time administered

The administering staff member observes the youth taking the medication before completing the MAR entry.

20.4 Medication Incidents

Incident	Required Response
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Refused dose	Document on MAR immediately with reason; notify supervisor; contact prescriber if repeated or clinically significant
Missed dose (staff error)	Document on MAR; notify supervisor; contact prescriber for direction; complete incident note in shift log
Vomited within 30 minutes	Document; notify supervisor; do not repeat without prescriber guidance
Wrong drug, wrong dose, wrong youth, or wrong time	Immediate medical concern: call prescriber and/or 8-1-1; call 911 if any adverse effect observed; complete Critical Incident Report within 2 hours; MCFD social worker notified same day

20.5 Controlled Substance Count at Every Shift Change

Outgoing and incoming staff jointly count all controlled substances at every shift change, reconcile against the previous count, sign the count log, and immediately report any discrepancy — even one dose — to the Program Manager. Discrepancies trigger an immediate investigation.

21. BEHAVIOUR SUPPORT AND POSITIVE PROGRAMMING

MCFD Standards C.6, D.7; CFCSA s.70; MCFD Policy 5.13

21.1 Philosophy

Behaviour is communication. When a young person in our care acts in ways that challenge us, they are almost always communicating something they cannot express any other way. Our response to that communication determines whether we are genuinely therapeutic or merely managing behaviour.

Evergreen Community Care's approach to behaviour support is grounded in trauma-informed, attachment-based, and developmentally responsive practice. We use the Behaviour Support Plan as the individualized clinical tool, and we use supervision, reflection, and continuous learning to improve our practice over time.

21.2 Positive Behaviour Support Practices — Required

These are the required approaches for all staff in all behaviour situations:

- Praise that is specific, genuine, and timely — not generic or sycophantic
- Structuring the environment and schedule to set youth up for success and minimize known triggers
- Teaching and modelling appropriate social behaviour through consistent adult example — how staff treat each other matters
- Responding to behaviour promptly, calmly, and without escalating
- Helping youth understand the connection between their choices and natural or logical consequences
- Maintaining the relationship through and after every difficult moment — the repair is part of the intervention

21.3 Prohibited Practices — Absolutely Forbidden

⚠ The following practices are prohibited by the CFCSA, MCFD Standards, and Evergreen Community Care policy under all circumstances. Any use of these practices is a Critical Incident requiring immediate MCFD notification and grounds for immediate disciplinary action:

- Any form of physical punishment: spanking, slapping, shaking, pinching, hitting, or any physical force used as a consequence
- Seclusion or confinement: placing a youth in any space from which they cannot exit without staff permission
- Withholding meals, bedding, or any basic physical need as a consequence
- Humiliation, ridicule, name-calling, or public shaming
- Unauthorized denial of family contact or communication
- Threatening removal or placement change to coerce compliance
- Physical restraint used as punishment, deterrent, or for staff convenience



- Any hold or technique not covered in current NVCI certification
- Prone (face-down) restraint under any circumstance

21.4 De-Escalation Framework

Stage	Signs to Watch For	Staff Response
Baseline (calm)	Regulated; engaged; within normal range	Relationship time; positive reinforcement; proactive BSP review
Agitation (early escalation)	Pacing; restlessness; voice change; rumination	Reduce demands; offer choice; increase space; activate BSP de-escalation strategies
Acceleration (escalating)	Verbal aggression; challenging language; property disruption	Calm, low-stimulation environment; call for backup; do not engage the content of threats; maintain safety for others
Crisis (active danger)	Physical aggression; active self-harm; risk of serious injury	Safety first; backup present; NVCI techniques if threshold met; 911 if needed
Recovery (de-escalating)	Reducing aggression; beginning to respond	Calm presence; do not debrief immediately; give time and space
Post-crisis (calm restored)	Youth is regulated and accessible	Welfare check; quiet reconnection; full debrief when youth is ready; document; review BSP



22. EDUCATION SUPPORT AND SCHOOL LIAISON

MCFD Standards D.6; Child in Care Regulation s.12; BC School Act

22.1 School Enrolment

- Every school-aged youth is enrolled in an appropriate educational program within 5 school days of placement
- School transfers are managed collaboratively; previous records including IEPs are requested and transferred immediately
- Where a youth's behaviour history requires a modified start, the Program Manager works with the school principal and MCFD social worker to develop a transition plan before the youth's first day
- Youth who are not enrolled in school have this documented in their ISP with the reason and an active plan for re-engagement

22.2 Attendance and Academic Support

- School attendance is tracked daily in the shift log; absences followed up the same day
- More than 2 unexcused absences in any 2-week period triggers a conversation with the school and notification to the MCFD social worker
- A consistent daily homework time and quiet workspace are provided
- Staff attend school meetings (parent-teacher, IEP reviews, case conferences) as part of the care team
- School suspensions are reported to the MCFD social worker as a Critical Incident on the same day

23. FAMILY CONTACT AND RELATIONSHIP MAINTENANCE

CFCSA s.70; MCFD Standards D.2, D.3; Child in Care Regulation

23.1 Principle

Family connection is a legal right of every child in care, not a privilege extended by caregivers. Staff do not allow their personal views about a family to influence the quality or frequency of support provided for family contact. Where contact is limited by the care plan or court order, the limitation is respected — and staff actively support the youth in coping with the emotional impact.

23.2 Family Contact Plan Requirements

Every youth's ISP includes a written Family Contact Plan specifying:

- Who is authorized for what type of contact (visits, phone, video, text)
- The frequency and format of authorized contact
- Any restrictions, with the documented reason for each restriction
- Any supervision requirements for visits
- The youth's own expressed preferences about contact

23.3 Facilitating and Documenting Contact

- Phone and video contact with authorized persons is facilitated promptly and privately when the youth requests it
- In-person visits are coordinated with the MCFD social worker; the home is welcoming and the environment is prepared
- All family contact is documented in the Contact Log: date, type, persons, duration, youth's mood before and after, and any concerns
- Significant concerns about the impact of contact on the youth's wellbeing are communicated to the MCFD social worker within 24 hours with documentation
- For Indigenous youth: regular contact with extended family, Nation, DAA, Elders, and cultural community members is actively facilitated as part of the Cultural Plan



24. COMMUNITY ACCESS AND LIFE SKILLS DEVELOPMENT

MCFD Standards D.4, D.5, D.6; Child in Care Regulation

24.1 Community Access as a Right

Youth in residential care have the right to participate in community life. Evergreen Community Care actively facilitates community access — it is not granted based on behaviour compliance. A community activities schedule is developed with each youth and documented in their ISP.

24.2 Life Skills Development Domains

Domain	Core Skills Developed	How We Support This
Home management	Cooking, cleaning, laundry, grocery shopping, home safety	Daily involvement in household tasks; graduated responsibility
Financial literacy	Budgeting, banking, understanding pay stubs, avoiding debt	Allowance management; supported shopping; practical math
Health and self-care	Hygiene, nutrition, sleep, exercise, accessing health care	Daily coaching; medical appointment accompaniment
Education and employment	Study skills, resume, job application, interview preparation	School support; employment programs; post-secondary exploration
Community navigation	Transit, government services, legal rights, emergency resources	Supported outings; real-world practice with guided reflection
Relationships and social skills	Healthy relationships, conflict resolution, consent, identifying safe adults	Role modelling; group discussions; individual coaching
Cultural connection	Language, practices, ceremony, community, cultural identity	Cultural Plans; Indigenous programming; Elder connections
Digital safety	Safe internet use, social media boundaries, online privacy	Supervised practice; age-appropriate discussion

24.3 Independent Living Preparation (Youth 16+)



For youth aged 16 and older, the ISP explicitly addresses preparation for independent living including: life skills assessment and targeted development; connection to Agreements with Young Adults (AWA) program; housing, income, and employment planning; and warm referral to Foundry, MCFD transition workers, and community supports before the youth's 18th birthday.



25. TRANSITION AND DISCHARGE PLANNING

MCFD Standards E.6; Contracted Service Agreement

25.1 Philosophy

Transitions — to another placement, back to family, or to independence — are among the highest-risk periods in a young person's care journey. Planning for the end of a placement begins at admission. Not because we are rushing anyone out, but because every young person deserves to know where they are going and to be supported to get there well.

25.2 Transition Planning Timeline

Timeframe	Required Action
At admission	Identify the intended type of discharge and a general timeline; document in the ISP
60 days before planned discharge	Formal Transition Planning meeting; Transition Plan (Appendix G) documented and shared with all parties
30 days before discharge	All transition tasks tracked; any blockers escalated to MCFD and Program Manager
2 weeks before discharge	Farewell planning with the youth; gradual orientation to new setting where possible
Day of discharge	Warm handover to receiving caregiver; youth leaves with all belongings, medications, and copies of key documents
Within 5 business days of discharge	Discharge summary provided to MCFD and receiving provider

25.3 Unplanned Discharges

When a placement is at risk: the Program Manager notifies the MCFD social worker at the earliest indication; an emergency care team meeting is convened within 48 hours; all available supports are explored before concluding the placement cannot continue; and if it must end, the youth is given maximum notice, maximum choice, and a warm handover wherever possible.

PART IV — SAFETY, INCIDENTS, AND CRITICAL CARE

26. CRITICAL INCIDENT REPORTING AND RESPONSE

MCFD Standards C.2, H.1; MCFD Critical Incident Reporting Policy; Contracted Service Agreement

26.1 Reporting Obligation

⚠ Failure to report a Critical Incident to MCFD is a serious breach of the Contracted Service Agreement and the CFCSA. All staff are personally responsible for ensuring incidents they witness are reported through the chain — but reporting to a supervisor does not relieve the obligation to ensure MCFD is notified within the required timeline.

26.2 Critical Incident Categories and Timelines

Category	Event Types	Timeline
Category 1 — Same Day	Death; hospitalization; abuse or neglect allegation; suicidal crisis or attempt; self-harm with injury; missing youth; police contact; physical restraint; traumatic exposure; school suspension	Verbal to MCFD social worker within 2 hours; written CI Report within 24 hours
Category 2 — Next Business Day	Marked behaviour change; medication error requiring medical attention; property damage over \$500; youth criminal activity; significant family crisis; unauthorized overnight	Written CI Report (Appendix D) by end of next business day
Category 3 — Weekly Report	Minor incidents with no immediate safety implication; pattern concerns requiring monitoring	Included in next weekly report to MCFD social worker

26.3 After-Hours MCFD Contact

If a Category 1 incident occurs and the social worker is unreachable: call MCFD Provincial Centralized Screening (PCS): 1-800-663-9122 (24 hours). Explain you are a residential care provider reporting a critical incident. Follow up with the social worker on the next business day.

26.4 Post-Incident Review

Within 5 business days of every Category 1 incident, the Program Manager reviews the incident, identifies contributing factors, documents corrective actions with named owners and target dates, and presents findings to the JHSC (where there are worker safety implications) and to MCFD. ISP and BSP are reviewed and updated within the same timeline.

27. SELF-HARM AND SUICIDAL CRISIS PROTOCOL

MCFD Standards C.1, C.2; MCFD Policy 5.9; BC Ministry of Health Suicide Prevention Framework

27.1 When to Activate This Protocol

This protocol activates when a youth discloses suicidal thoughts or plans; is found engaging in self-harm; makes statements or displays behaviour that staff reasonably interpret as suicidal ideation or intent; or has recent access to means combined with a significant recent loss or destabilizing event. Staff do not wait for certainty. Concern is sufficient to activate.

27.2 Immediate Response Sequence

Step	Required Action
1. Stay present	Remain with the youth at all times; do not leave them alone; call for backup while staying
2. Remove means	Remove access to identified means if possible without escalating; do not engage in a physical struggle
3. Engage calmly	Quiet, calm, non-alarming voice; genuine care without panic
4. Ask directly	“Are you thinking about ending your life?” — direct questions do not increase risk; they open conversation
5. Listen	Allow the youth to speak; do not reassure prematurely, minimize, or problem-solve
6. Call supervisor	Notify on-call supervisor immediately
7. Call 911 if needed	If the youth has harmed themselves; taken an overdose; has a weapon or active means; or is in immediate danger of serious injury
8. Notify MCFD	Call MCFD social worker or PCS (1-800-663-9122) within 2 hours regardless of how the crisis resolved

27.3 Post-Crisis Actions

- Critical Incident Report within 24 hours
- Safety Plan reviewed and updated with MCFD social worker within 24 hours
- Environmental risk assessment: remove or secure identified means per updated safety plan
- Mental health follow-up confirmed within 48 hours
- EFAP and debrief offered to all involved staff before end of shift



- ISP and BSP reviewed within 5 business days
- Crisis resources kept available for the youth: Crisis Centre BC (1-800-784-2433); Kids Help Phone (1-800-668-6868 or text HOME to 686868)



28. MISSING YOUTH PROTOCOL

MCFD Missing Child/Youth Protocol (revised May 2025); PCS: 1-800-663-9122

28.1 Phase 1 — Prudent Parent Response (Maximum 30 Minutes)

Before activating the full protocol, staff simultaneously:

- Call the youth's cell phone; text; try messaging apps they use
- Contact anyone who may know: named friends in the ISP; family on the contact list; school contacts if a school day
- Search the entire facility and grounds
- Check known community locations the youth frequents
- Notify the on-call supervisor and begin the chronological missing youth log

If at any point there is reason to believe the youth's safety is in immediate danger, skip Phase 1 and call 911 immediately.

28.2 Phase 2 — Missing Youth Confirmed

Action	By Whom / When
Notify MCFD social worker or PCS (1-800-663-9122)	Shift lead; immediately
Call 911 / RCMP if: immediate safety concern; social worker directs; or no direction after 1 hour	Shift lead
Notify Program Manager	Shift lead; within 30 minutes
Continue chronological log of all contacts and responses	Both on-shift staff
Notify Executive Director	Program Manager; same day if missing over 2 hours
Critical Incident Report	Program Manager; within 24 hours regardless of outcome

⚠ NEVER disclose to police, media, or the public that the youth is in government care under the CFCSA. This is a privacy breach under FOIPPA. State only that they are a resident of your program.

28.3 When the Youth Returns

- Physical and emotional wellness check immediately — warm, non-punitive, without interrogation



- Offer food, warmth, and calm space before any structured conversation
- Notify police (if involved), MCFD social worker, and Program Manager immediately
- Debrief with the youth within 24 hours when they are ready
- ISP and safety plan reviewed within 5 business days

29. PHYSICAL RESTRAINT POLICY AND POST-INCIDENT RESPONSE

MCFD Standards C.3; MCFD Policy 5.13; CFCSA s.70; OHS Regulation s.4.27–4.31

29.1 Position Statement

⚠ Physical restraint is a measure of last resort used ONLY to prevent immediate risk of serious physical harm. It is never used as a consequence, punishment, or for staff convenience. Every use of physical restraint is a Critical Incident requiring immediate MCFD notification.

29.2 Authorization Criteria — All Must Be Met Simultaneously

- There is an imminent risk of serious physical harm — not disruptive behaviour or refusal
- Less restrictive means have been attempted or the immediacy of the risk makes them impractical
- The staff member is currently NVCI-certified
- The restraint uses the minimum force necessary for the minimum time necessary

29.3 Absolutely Prohibited Practices — No Exceptions

Prohibited Practice	Authority
Prone (face-down) restraint	MCFD Policy 5.13 — absolute prohibition
Any hold restricting breathing, blood flow to the brain, or causing chest compression	Life-threatening; absolute prohibition
Pressure on neck, throat, or abdomen	Absolute prohibition
Seclusion: preventing a youth from leaving any space	MCFD Standards C.3; CFCSA s.70
Restraint used as punishment, deterrent, or consequence	Violates CFCSA s.70 and MCFD Standards C.3
Any technique not covered in current NVCI certification	OHS Reg. s.4.27–4.31
Leaving a restrained youth unattended for any reason	MCFD Standards C.3



29.4 Mandatory Post-Restraint Actions — All Required

Action	Timeline
Medical wellness check of youth	Immediately on release
911 for any injury, pain complaint, breathing concern, or loss of consciousness	Immediately
Notify on-call supervisor	Immediately
Debrief with the youth when regulated	Same shift if possible; within 24 hours
Debrief all involved staff	Before end of shift
Notify MCFD social worker or PCS	Within 2 hours
Offer EFAP to all involved staff	Before end of shift
Complete Critical Incident Report	Within 2 hours
JHSC worker safety notification	Within 24 hours
ISP and BSP review	Within 5 business days



30. EMERGENCY PREPAREDNESS FOR RESIDENTIAL CARE

MCFD Standards F.4; OHS Regulation; BC Fire Code; see also OHS Program Binder (ECC-OHS-POL-2026-002)

30.1 Emergency Procedures Specific to Residential Care

Emergency	Residential Care-Specific Modifications
Fire evacuation	Evacuation plan accounts for all youth and staff by name; youth who resist evacuation receive verbal guidance then physical guidance (not restraint); Area of Rescue Assistance identified for youth who cannot self-evacuate; staff role assignments confirmed at start of each shift
Medical emergency	911 called immediately; staff member accompanies youth to hospital; MCFD social worker notified within 1 hour; one staff remains with other youth at the facility
Youth-initiated violence	Other youth removed from the immediate area first; de-escalation attempted; backup called; 911 if situation cannot be safely managed; MCFD notified same day
Security threat / unauthorized entry	Lock all exterior doors; secure youth in interior areas; call 911; do not open doors until police confirm all-clear; MCFD notified same day
Power outage (extended)	Flashlights activated; temperature monitored; refrigerated medications assessed; MCFD notified if outage exceeds 4 hours
Missing youth during an emergency	Location communicated to fire/police immediately; staff do not re-enter an unsafe structure to search

30.2 Required Emergency Contacts Posted at Every Facility

- 911 (police, fire, ambulance)
- MCFD Provincial Centralized Screening: 1-800-663-9122 (24 hours)
- On-call supervisor: [current schedule posted weekly]
- Program Manager: [number posted]
- Poison Control: 1-800-567-8911
- Crisis Centre BC: 1-800-784-2433
- Advocate for Children and Youth: 1-800-476-3933
- Helpline for Children: 1-800-663-9122
- Facility address: [posted at every telephone]

30.3 Emergency Drill Schedule



Drill Type	Frequency
Fire evacuation (announced)	Quarterly at each facility
Fire evacuation (unannounced — night or weekend shift)	Annually
Medical emergency tabletop (team meeting)	Annually
Missing youth protocol review	Annually; and following any missing youth event
Lockdown / security threat tabletop	Annually

PART V — CULTURAL SAFETY AND EQUITY

31. INDIGENOUS RECONCILIATION FRAMEWORK AND CULTURAL SAFETY PLAN

DRIPA (SBC 2019, c.44); CFCSA s.2, 70, 71; TRC Calls to Action; UNDRIP; MCFD Standards G.3

31.1 Organizational Commitment

Evergreen Community Care acknowledges with accountability that Indigenous children are vastly overrepresented in government care — a direct consequence of colonization, the residential school system, the Sixties Scoop, and systemic racism in child welfare. We are committed to being part of the solution: through active cultural safety practices, genuine Nation relationships, and daily care that affirms and nurtures Indigenous children's identity.

This commitment is guided by the Declaration on the Rights of Indigenous Peoples Act (DRIPA), the TRC Calls to Action (particularly Call to Action 65 on Indigenous child welfare), and UNDRIP Articles 7, 8, 14, and 21.

31.2 Indigenous Placement Principles

When residential care placement is required for an Indigenous child or youth, the following placement preference order applies and is actively pursued before accepting a group care placement:

Priority	Placement Type
First	Extended family of the child — Indigenous family members
Second	Extended family or persons nominated by the child's family
Third	A family approved by the child's Nation or Indigenous community
Fourth	Another Indigenous family
Fifth	A non-Indigenous family committed to actively supporting the child's cultural identity
Group care (including Evergreen)	Only when family-based options have been genuinely explored and are not currently available

31.3 Cultural Plan Requirements

Every Indigenous child or youth has a written Cultural Plan developed within 30 days of placement, in genuine collaboration with the youth, their family, and their Nation. The Cultural Plan (Appendix H) documents: Nation and community affiliation; the youth's own goals for cultural connection; specific activities, ceremonies, and community participation; Elders and



cultural mentors involved; traditional foods, medicines, and healing practices; language learning supports; DAA involvement; and the responsibilities of Evergreen Community Care, MCFD, and the Nation.

31.4 Staff Cultural Safety Obligations

- All staff complete Indigenous Cultural Safety and Humility training (FNHA-recognized or equivalent) within 60 days of hire and annually thereafter — this is mandatory, not optional
- Smudging, ceremony, drumming, and other cultural practices are accommodated in the home without judgment or restriction
- Cultural foods are incorporated into meal planning as requested; the program budget accommodates this
- Any racially discriminatory comment or action by any person — staff, volunteer, or other youth — is addressed immediately, documented, and reported to MCFD
- Staff examine their own assumptions and biases continuously; cultural humility is a career-long practice, not a one-time training outcome



32. GENDER-AFFIRMING CARE PROGRAM

MCFD Standards C.1.1(b), D.1.5(b), D.1.13(b), D.6(g), G.3.2(b), G.4.3(c) — all amended April 2024

32.1 MCFD Standard and Organizational Commitment

Gender-affirming care is defined by MCFD as: the processes through which caregivers, staff, services, and organizations care for and support an individual, while recognizing and acknowledging their gender identity and expression to promote their safety, well-being, and belonging. This is a mandatory MCFD Standard — not a program option.

32.2 Core Obligations — All Staff

Obligation	Detail
Correct name and pronouns	Every staff member uses each youth's stated name and pronouns consistently from the first day; deliberate misgendering is a Code of Ethics violation subject to disciplinary action
Affirm identity and expression	Youth's gender identity is supported, affirmed, and nurtured; staff do not question, pathologize, or express discomfort
Confidentiality	A youth's gender identity is personal and confidential; not shared without informed consent except where a documented safety reason requires disclosure
Mandatory training	All staff complete gender-affirming care training within 60 days of hire and annually thereafter
Respond to discrimination	Any misgendering, harassment, or discrimination from any person — staff, youth, or visitor — is addressed immediately, documented, and treated as an incident

32.3 Practical Accommodations

- Bedroom and bathroom arrangements reflect the youth's gender identity wherever operationally feasible and with the youth's input
- Youth have access to gender-affirming clothing, personal care items, and hygiene supplies that reflect their gender identity
- Youth have access to gender-affirming mental health resources and LGBTQ2S+ community organizations
- Gender-affirming care needs are documented in the ISP and reviewed at every ISP review



33. ANTI-RACISM AND EQUITY PRACTICE

BC Human Rights Code; CFCSA s.2; DRIPA; MCFD Standards

33.1 Commitment

Evergreen Community Care is committed to actively anti-racist organizational culture and practice. Racism is not only individual prejudice — it is also systemic, structural, and embedded in practices that cause harm even without conscious intent. We commit to examining and changing both individual behaviours and organizational structures that perpetuate inequity.

33.2 Staff Responsibilities

- Model respectful, anti-racist behaviour in all interactions with youth, families, colleagues, and community partners
- Respond immediately and directly to any racist comment, joke, or behaviour from any person in the program — including from other youth. Silence is not neutral.
- Examine personal biases through training, supervision, and reflection; cultural humility is ongoing
- Document and report racism experienced in or by the program through the incident reporting process

33.3 Organizational Actions

- Annual review of practices for disparate impact on racialized youth
- Active effort to recruit and retain staff from Indigenous, racialized, and equity-seeking communities
- Physical environment reflecting the cultural diversity of youth served — artwork, images, books, food, and media
- Anti-racism and equity training within 60 days of hire and every 2 years thereafter
- Immediate, documented response when a youth reports experiencing racism — validated, supported, and escalated to MCFD within 24 hours



34. TRAUMA-INFORMED CARE FRAMEWORK

MCFD Standards D.6, G.3; BC Trauma-Informed Practice Guidelines; SAMHSA TTIC Framework

34.1 Definition

Trauma-informed care is not a program or technique. It is an organizational orientation that recognizes the widespread impact of trauma on the young people we serve, integrates that knowledge into every policy and practice, and actively avoids re-traumatization while supporting healing. At Evergreen Community Care, trauma-informed care is how we operate — not something we do in addition.

34.2 SAMHSA's Six Foundations — Applied to Evergreen

Foundation	How We Apply It
Safety	Physical and psychological safety in every shift; predictable routines; transparent communication; no surprises in consequences or expectations
Trustworthiness and transparency	Staff are honest even when the truth is hard; promises are kept; decisions are explained; youth know what is happening and why
Peer support	Youth are supported to connect with and support each other; peer relationships are therapeutic resources, not just management challenges
Collaboration and mutuality	Care is done with youth, not to them; power differentials are acknowledged; youth are given real choice and real voice
Empowerment and choice	Youth have choices in every aspect of their care; their strengths are recognized and built upon; they develop mastery through graduated responsibility
Cultural, historical, and gender responsiveness	Cultural safety and gender affirmation are not add-ons to trauma-informed care; they are integral to it

34.3 Secondary Traumatic Stress and Staff Wellbeing

Staff who care for traumatized youth are themselves at significant risk of secondary traumatic stress and compassion fatigue. Evergreen Community Care addresses this as an organizational responsibility, not an individual failing:

- Reflective supervision minimum monthly for all direct care staff; structured to include emotional processing, not just task management
- Team debriefs offered within 48 hours of every significant critical incident



- EFAP actively promoted from the first day of employment; number posted in every staff area
- Scheduling decisions take emotional and clinical load into account, not only hours worked
- A culture where asking for help, admitting difficulty, and seeking support is respected and never penalized

PART VI — STAFFING AND PROGRAM GOVERNANCE

35. STAFFING RATIOS AND DEPLOYMENT

Residential Care Regulation s.66–72; MCFD Standards G.2; Contracted Service Agreement

35.1 Minimum Staffing Standards

Staffing ratios are governed by the Residential Care Regulation, the facility licence, and the Contracted Service Agreement. Where these differ, the highest standard prevails. No shift operates below licensed minimum ratios under any circumstances.

Shift Type	Minimum Requirement	Enhanced Triggers
Daytime / evening shift	Per licence and Contracted Service Agreement	Any youth with active safety plan requiring enhanced supervision; two or more high-acuity youth simultaneously
Overnight shift	Minimum one awake, on-site staff responsible for all youth at all times	Youth with documented self-harm or elopement risk; acute mental health crisis
Community outings	Per Appendix K outing safety plan	1:1 for youth with active self-harm or elopement risk; 2 staff for swimming or high-risk activities

35.2 Below-Minimum Staffing Protocol

If staffing falls below minimum: shift lead notifies Program Manager immediately; Program Manager takes all reasonable steps to arrange coverage within 2 hours; if minimum cannot be restored, MCFD social worker is notified and a Critical Incident Report is completed documenting all steps taken.

35.3 Shift Handover Requirements

- Minimum 15-minute in-person overlap between outgoing and incoming shift lead
- Verbal handover covers: location and status of every youth; mood and presentation; incidents or concerns from the shift; medications administered and any exceptions; controlled substance count reconciled; outstanding tasks for incoming shift
- Written handover: shift log completed and signed before outgoing staff depart
- Incoming shift lead confirms all critical information received before outgoing staff leave



36. STAFF ORIENTATION AND TRAINING PROGRAM

MCFD Standards G.3; HR Policy Binder; OHS Regulation s.3.22–3.23

36.1 Mandatory Training Matrix

Training	Audience	Initial Timeline	Renewal
Residential Care Program Manual orientation	All staff	Before first independent shift	Review at each annual update
Youth rights and mandatory reporting (CFCSA s.14)	All staff	Before first shift	Every 2 years
Non-Violent Crisis Intervention (NVCI)	All direct care staff	Within 90 days; Module 1 before solo shifts	Annual
Indigenous Cultural Safety and Humility	All staff	Within 60 days	Annual
Gender-Affirming Care	All staff	Within 60 days	Annual
Trauma-Informed Care foundations	All direct care staff	Within 60 days	Every 2 years
Suicide and self-harm response (ASIST or equiv.)	All direct care staff	Within 60 days	Annual
Medication Administration	Authorized staff only	Before any medication duty	Annual competency
First Aid / CPR Level C	All direct care staff	Within 30 days	Annual
OFA Level 2 (designated attendant per shift)	Minimum 1 per shift	Before first duty as designated attendant	Per certification cycle
WHMIS 2015	All staff	Before first shift	Every 3 years
Working Alone safety	Staff on solo overnight shifts	Before first solo overnight	Annual
Driver safety (with annual abstract)	Any driver of youth	Before first driving duty	Annual abstract review

36.2 Training Compliance Monitoring



- Training compliance is tracked in the Staff Training Compliance Matrix (Appendix L), updated monthly by the Program Manager
- Staff with overdue mandatory training are removed from duties requiring that training until compliance is restored
- Training costs are borne by the organization; staff are paid at their regular rate for training time



37. SUPERVISION AND REFLECTIVE PRACTICE

MCFD Standards G.4; HR Policy Binder; best practice in child and youth care

37.1 Individual Supervision Structure

Element	Standard
Frequency	All direct care staff: minimum monthly. New staff (first 90 days): minimum every 2 weeks. Staff on performance improvement plans: every 2 weeks. Program Managers: supervised by Executive Director minimum monthly.
Duration	Minimum 60 minutes; uninterrupted
Format	In-person where possible; scheduled with protected time
Confidentiality	Private; notes retained in personnel file; accessible only to the supervisor and staff member

37.2 Required Supervision Content

Content Area	What Is Explored
Emotional check-in	How is the staff member doing? Signs of secondary traumatic stress or compassion fatigue?
Case review	Specific youth discussed: what is going well; what is challenging; what ISP strategies are working
Practice reflection	A recent significant interaction examined: internal experience; response choices; what might be done differently
Critical incident review	Any incident since last supervision: was the response appropriate; what does the staff member need to process?
Professional development	What is the staff member working on? What support or training do they need?
Honest feedback	Specific, two-way; includes genuine recognition of strengths and clear identification of areas for growth

37.3 Team Meetings



Monthly team meetings are mandatory and include: case consultation (reflective group discussion of one or more youth); critical incident learnings; practice update or training component; team wellbeing; organizational updates. Staff are encouraged to raise practice dilemmas and ethical concerns; the forum is psychologically safe.



38. PROGRAM REVIEW AND QUALITY ASSURANCE

MCFD Standards H.2; Contracted Service Agreement; QA Program Binder (ECC-QA-2026-001)

38.1 QA Calendar

Frequency	Activity	Responsible
Every shift	Shift safety check; shift log completed; medication count reconciled	Shift lead
Monthly	Facility safety inspection; training compliance review; CI incident trend review; individual supervision completed for all staff	Program Manager
Quarterly	CI trend analysis report; ISP file audit (minimum 25% sample); KPI dashboard reviewed	Program Manager
Biannually	Full ISR file audit; Youth and Family Feedback Survey	Program Manager
Annually	MCFD Standards A-H self-assessment; Staff Satisfaction Survey; Annual Program Review; full manual review and update	Program Manager + Executive Director

38.2 Key Performance Indicators

KPI	Target
CI reports submitted to MCFD within required timelines	100%
ISP reviews completed on schedule	100%
Staff with all mandatory training current	100%
Physical restraint incidents	Year-over-year reduction; zero tolerance for any prohibited technique
Youth school attendance (enrolled youth)	85%+ attendance; 100% enrolled
Youth satisfaction (annual survey)	70%+ satisfied or very satisfied



Placement stability (beyond 90 days)	75%+ placements stable
Licensing deficiencies	Zero critical deficiencies; all others remediated within required timelines

38.3 Version History

Version	Date	Summary
1.0	January 2025	Initial policy manual — 15 sections
2.0	July 1, 2026	Comprehensive revision — all MCFD Standards A–H; April 2026 RCR amendments; gender-affirming care; missing youth protocol updates
3.0	July 1, 2026	Full program manual — 30 sections + 12 appendices; Parts III–VI operational standards added
4.0	July 1, 2026	Reformatted to match v3 source document styling; Mission and Values completely rethought (s.4); all operational content integrated; version numbering consolidated



APPENDIX A — ADMISSION AND INTAKE CHECKLIST

Complete at admission. Both admitting staff member and Program Manager sign. File in Individual Service Record. Retain 7 years minimum.

Field	
Youth Legal Name	
Preferred Name / Pronouns	
Date of Birth	
Date of Admission	
Legal Status	
MCFD Social Worker and Phone	
Admitting Staff Member	
Program Manager	

Admission Task	Done?	Notes / Action Required
MCFD placement authorization confirmed	☐	
Pre-admission assessment completed by Program Manager	☐	
Facility capacity confirmed	☐	
Youth welcomed; bedroom assigned and shown	☐	
Personal belongings inventoried and stored	☐	
Rights information reviewed verbally;	☐	



Appendix C handout provided		
Advocate contact number given: 1-800-476-3933	☐	
Complaint process explained	☐	
Medical information and health alerts confirmed	☐	
All medications received, inventoried, and locked; MAR initiated	☐	
Controlled substance count completed with both shift staff	☐	
Hot water temperature at all taps confirmed $\leq 49^{\circ}\text{C}$ (RCR April 2026)	☐	
Court orders and contact restrictions reviewed	☐	
Environmental risk assessment reviewed for this youth's risk profile	☐	
Indigenous status confirmed; Cultural Plan meeting scheduled	☐	
Preliminary ISP documented and in use within 48 hours	☐	
Full ISP meeting scheduled within 14 days	☐	
Youth orientation	☐	



completed (Appendix B)		
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Program Manager Signature / Date	
Admitting Staff Signature / Date	



APPENDIX B — NEW YOUTH ORIENTATION GUIDE

Staff guide for welcoming a new young person. Use as a conversation guide — not a script. Go at the youth's pace.

Welcome

"Welcome to [program name]. My name is [staff name]. I want to show you around and tell you how things work here. There are no stupid questions — ask me anything."

The Tour

Walk at their pace: their bedroom and how it's their private space; their personal locked storage; bathroom(s); kitchen and meal times; common areas; outdoor space; all exits and the fire assembly point; emergency contacts poster; staff workspace.

Rights

"You have rights in this home. Real ones. The most important ones right now: no one can physically punish you for any reason. You can always contact your social worker, the Advocate (1-800-476-3933), or the police — in private, without asking permission. And you can make a complaint if something is wrong, and nothing bad will happen to you for doing it."

Daily Routine

Cover: wake-up and bedtime; meal times and who cooks; school schedule; chores; activity and free time; who works which shifts.

Expectations

Keep it brief and warm: "We want this to feel like a real home. We just need everyone — including staff — to treat each other with respect. Here's what that looks like practically..."

Questions

"That's a lot of information. Do you have any questions right now? You can always come back to me or any staff member — any time."

Orientation completed by: _____ Date: _____

Youth acknowledged (or reason documented if declined): _____



APPENDIX C — RIGHTS INFORMATION HANDOUT — YOUTH VERSION

Provide at admission. Post in the common area. Available in other languages and formats on request.

YOUR RIGHTS — THESE BELONG TO YOU

These are legal rights. No one can take them away, withhold them as punishment, or use them as rewards.

- You have the right to be **SAFE**. No one can hit you, physically punish you, or threaten you.
- You have the right to know what is happening in your life and to have a say in decisions about you.
- You have the right to **PRIVACY** and to keep your own things safely.
- You have the right to contact **FAMILY** — within your care plan.
- You have the right to go to **SCHOOL** and to receive medical care when you need it.
- You have the right to your **CULTURE**, language, and community.
- You have the right to have your **GENDER IDENTITY** respected by everyone here.
- You have the right to make a **COMPLAINT** — and nothing bad will happen to you for speaking up.
- You have the right to contact **OUTSIDE PEOPLE** in private, at any time, without permission.

Who to Call	Number
Emergency — Police / Fire / Ambulance	911
Helpline for Children (24/7)	1-800-663-9122
Advocate for Children and Youth	1-800-476-3933
BC Ombudsperson	1-800-567-3247
Kids Help Phone (24/7)	1-800-668-6868 or text HOME to 686868
Crisis Line BC (24/7)	1-800-784-2433
Your Social Worker	Name: _____ Phone: _____
This Program's Manager	Name: _____ Phone: _____



APPENDIX D — CRITICAL INCIDENT REPORT FORM

Complete for every Category 1 and Category 2 Critical Incident. Verbal notification to MCFD precedes this form. File in youth's ISR. Retain permanently.

Field	
Youth Name (Legal and Preferred)	
Date and Time of Incident	
Location	
Report Completed By (name and title)	
Date and Time Report Completed	
Supervisor Notified — Name and Time	
MCFD Social Worker Notified — Name and Time	
911 Called?	☐ Yes — Police ☐ Yes — Ambulance ☐ Yes — Fire ☐ No
Incident Category	☐ Category 1 — same-day verbal + written within 24 hrs ☐ Category 2 — written by next business day
Incident Type	☐ Injury/medical ☐ Self-harm ☐ Suicidal crisis ☐ Missing youth ☐ Physical restraint ☐ Abuse allegation ☐ Police contact ☐ School suspension ☐ Medication error ☐ Other: ____

Section	Detail (factual and objective — what was observed, not interpreted)
1. Chronological description of the incident	
2. Youth's own account (their words)	
3. Immediate actions taken by staff	
4. Youth's physical and emotional status following	



the incident	
5. Contributing or contextual factors	
6. Corrective actions required (owner + target date for each)	
7. ISP / BSP review triggered? Y / N Scheduled for:	

Completing Staff Signature / Date	Program Manager Signature / Date	MCFD SW Acknowledgment / Date



APPENDIX E — BEHAVIOUR SUPPORT PLAN TEMPLATE

Develop collaboratively with the youth. Review at every ISP review. Update within 5 days of any significant behaviour incident.

Youth Name	
Date of Birth	
BSP Author	
Date Developed	
Review Date	
MCFD Social Worker	

Section 1 — Understanding This Youth

Element	Detail (developed with the youth's input)
What this youth is good at; their strengths and interests	
What matters most to this youth (people, activities, identity, goals)	
Known triggers — what tends to lead to escalation for this youth specifically	
Early warning signs specific to this youth	
What the youth says helps when they are struggling	
What does NOT help (and may make things worse)	
Key people who ground and regulate this youth	
Cultural, spiritual, or identity factors relevant to this youth's care	

**Section 2 — De-escalation Strategies (in sequence for THIS youth)**

Stage	Staff Response for This Youth Specifically
Early signs of dysregulation	
Active escalation	
Crisis (potential harm to self or others)	
Post-crisis recovery	

Section 3 — Authorized Interventions

Document only interventions agreed with MCFD and for which staff are trained:

Intervention	Conditions for Use	Staff Authorization
Verbal de-escalation (all staff)	Always the first response	All staff
NVCI-authorized supportive hold (if applicable)	Imminent physical harm only; see Section 29	Current NVCI certification
Other (describe):		

Youth Input Documented / Date	Program Manager Signature / Date	MCFD Social Worker Signature / Date



APPENDIX F — MEDICATION ADMINISTRATION RECORD (MAR)

Complete a separate MAR each calendar month. Document every dose — administered, refused, or missed — immediately. Retain 10 years minimum.

Youth Name	
Date of Birth	
BC Personal Health Number	
Allergies (ALERT — list all)	
MAR Month / Year	
Prescribing Physician(s)	
Dispensing Pharmacy	

Administration Key: Write initials + time for each dose administered. R + time for refusals. M + reason for missed doses. All exceptions require a note in the shift log.

Medication / Dose / Route / Instructions	Time(s) per Day	Days 1–15 (initials + time)	Days 16–31 (initials + time)

Initials Key — all staff who administered medications this month sign below:

Initials	Full Name (Printed)	Signature
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Program Manager monthly review — Signature / Date:



APPENDIX G — SAFE TRANSITION PLANNING CHECKLIST

Initiate at minimum 6 months before planned transition. Review monthly until discharge. File in ISR.

Youth Name	
Planned Transition Date	
Destination	
MCFD Social Worker	
Transition Plan Meeting Date	

Transition Task	Done?	Notes
Youth meaningfully involved in transition planning	☐	
Youth's goals and concerns about the transition documented	☐	
Youth knows where they are going, when, and why	☐	
Transition Plan documented in ISP	☐	
Receiving provider has: full ISP; health file; medication list	☐	
Receiving provider has: BSP; Cultural Plan (if applicable)	☐	
School transfer arranged; new school has records and IEP	☐	
New family physician identified;	☐	



appointment made		
Medication supply (minimum 2 weeks) provided	☐	
All personal belongings inventoried and packed	☐	
Youth has personal ID documents (birth certificate, PHN, SIN)	☐	
Cultural connections confirmed in new setting	☐	
Community supports confirmed (mental health, recreation, employment)	☐	
For youth aging out: AWA explored; housing confirmed; income supports	☐	
Farewell activity planned per youth's preference	☐	
Discharge summary completed within 5 business days	☐	

Program Manager / Date	MCFD Social Worker / Date	Youth (if willing) / Date



APPENDIX H — CULTURAL PLAN TEMPLATE FOR INDIGENOUS YOUTH

Develop within 30 days of placement with the youth, their family, and their Nation. Update at every ISP review.

Field	
Youth Name	
Nation / Band / Community	
DAA (if applicable)	
Cultural Support Worker	
Date Developed	
Review Date	

Cultural Plan Element	Detail (developed with the youth's active input)
Youth's current relationship with their cultural identity and community	
Youth's own goals for cultural connection during this placement	
Indigenous Nation, language group, and any band affiliation	
Family and community people who support the youth's cultural connection	
Elders, Knowledge Keepers, and cultural mentors to be involved	
Specific cultural activities, ceremonies, and practices to be facilitated	
Traditional foods to be incorporated into meal planning	



Traditional medicines or healing practices to be supported	
Language learning: Is the youth interested? Resources identified:	
DAA contact and involvement frequency	
Friendship Centre or urban Indigenous organization connections	
Responsibilities of Evergreen Community Care in implementing this plan	
Responsibilities of MCFD in supporting this plan	
Responsibilities of the Nation/community in supporting this plan	
How the youth and their Nation will know this plan is working	

Youth (if willing) / Date	Family / Community Rep / Date	MCFD Social Worker / Date	Program Manager / Date



APPENDIX I — YOUTH COMPLAINT AND FEEDBACK FORM

You can use this form to tell us if something is wrong or to give us feedback. You can also talk to any staff member, your social worker, the Advocate (1-800-476-3933), or the Ombudsperson at any time. You will not get in trouble for using this form.

Your Name (you can stay anonymous)	
Date	
Your concern is about:	☐ A staff member ☐ Another youth ☐ The program or rules ☐ Something in the home ☐ Other

What happened? (in your own words):

What would you like to happen?

Did you feel safe raising this concern? ☐ Yes ☐ No ☐ Not sure

For Staff Use — complete same day	
Received by (name and date)	
Acknowledged to youth on / how	
Program Manager notified on	
Investigation completed by / date	
Response provided to youth on / how	
Escalated to Executive Director? Y / N	



APPENDIX J — MONTHLY PROGRAM QUALITY REVIEW TOOL

Completed by Program Manager. File with QA Program Binder. Drives continuous improvement.

Facility / Program	
Month / Year	
Reviewer	
Date Completed	

Quality Indicator	Result / Status	Action Required / Owner / Date
Total Critical Incidents this month (by category)		
All CI reports to MCFD within required timelines? Y/N		
Same youth or same type CI as last month? Trend?		
Physical restraints this month: number and circumstances		
All ISPs reviewed on schedule? Y/N		
BSPs updated following any incident? Y/N		
Medication count discrepancies this month		
Medication errors this month: number and type		



Shift log quality (sample review): rating 1–5		
All mandatory training current for all staff? Y/N; gaps:		
Individual supervision completed for all staff? Y/N		
Team meeting held? Y/N Date:		
School attendance: % of scheduled days attended		
Family contact plan followed? Issues?		
Community outings completed per ISP plans? Y/N		
Youth complaints received: number and category		
Facility safety inspection completed? Y/N Deficiencies:		
Any shifts below minimum staffing? Y/N Circumstances:		
Open corrective actions from last month: status		
Cultural Plan activities completed per plan? Y/N		

Program Manager Summary — Key Findings and Priorities for Next Month:



Program Manager Signature: _____ Date: _____

**APPENDIX K — TRANSPORT AND COMMUNITY OUTING SAFETY PLAN**

Complete before any off-site activity. File in shift log. Retain for 1 year.

Activity / Destination	
Date	
Departure Time	
Expected Return Time	
Staff Lead	
Additional Staff	
Youth Attending (names)	
Transport Method	

Safety Check	Done?	Notes
Each youth's ISP reviewed for activity restrictions or supervision requirements	☐	
Staff-to-youth ratio meets requirements for each youth attending	☐	
Emergency medications brought (EpiPen, puffer, seizure medication)	☐	
Emergency contacts for all youth in staff bag or phone	☐	
Facility number and on-call supervisor number confirmed	☐	



Vehicle inspected; seatbelts and child restraints confirmed	☐	
Destination assessed for hazards (water, traffic, crowds)	☐	
Head count completed before departure	☐	
Head count completed on return to facility	☐	
Outing documented in shift log on return	☐	

Staff Lead Signature / Date	
Program Manager Review (if incident) / Date	



APPENDIX L — STAFF TRAINING COMPLIANCE MATRIX

Updated monthly by Program Manager. Staff with expired mandatory training are not assigned duties requiring that training.

Facility / Program	
Month / Year	
Updated By	

Training Requirement	Staff 1	Staff 2	Staff 3
NVCI — expiry date			
Indigenous Cultural Safety — expiry			
Gender-Affirming Care — expiry			
Trauma-Informed Care — expiry			
Suicide/Self-Harm Response — expiry			
Mandatory Reporting — expiry			
Medication Admin — expiry (if authorized)			
First Aid CPR Level C — expiry			
OFA Level 2 — expiry (designated attendant)			
WHMIS 2015 — expiry			
Working Alone — expiry (solo overnight staff)			
Driver abstract — date on file			

Training Requirement	Staff 4	Staff 5	Staff 6
NVCI — expiry date			



Indigenous Cultural Safety — expiry			
Gender-Affirming Care — expiry			
Trauma-Informed Care — expiry			
Suicide/Self-Harm Response — expiry			
Mandatory Reporting — expiry			
Medication Admin — expiry (if authorized)			
First Aid CPR Level C — expiry			
OFA Level 2 — expiry (designated attendant)			
WHMIS 2015 — expiry			
Working Alone — expiry (solo overnight staff)			
Driver abstract — date on file			

Enter date of completion or expiry in each cell. Highlight any expired or expiring within 30 days.

Program Manager Signature: _____ Date: _____

15. SIGNATURES AND APPROVAL

This policy manual has been developed and approved in good faith to meet or exceed all current MCFD residential care standards and licensing requirements.

Executive Director — Signature / Date

Program Manager, Residential Care — Signature / Date

Board Chair (or Delegate) — Signature / Date

Date Adopted